POLICY AND PROCEDURE MANUAL

Residential habilitation centers

[AGENCY NAME]

Oklahoma

Version 1.0

Effective Date: [Effective Date]

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DISCLAIMER: This Policy and Procedure Manual is provided as a template and guide for Residential habilitation centers operations in Oklahoma. While every effort has been made to ensure accuracy and compliance with current regulations, this document should be reviewed and customized by qualified personnel before implementation.

Regulatory requirements are subject to change. [AGENCY NAME] is responsible for ensuring ongoing compliance with all applicable state and federal regulations. This manual does not constitute legal advice. Consult with legal counsel and regulatory experts for specific compliance questions.

All placeholders marked with [BRACKETS] must be replaced with agency-specific information before this manual is put into use.

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Introduction

Welcome Statement

Welcome to the Policy & Procedure Manual for Oklahoma Residential Habilitation Centers. This manual serves as a comprehensive resource for [AGENCY NAME], dedicated to providing high-quality care and support for individuals with developmental disabilities. We understand the unique challenges and responsibilities that come with this vital role, and it is our hope that this manual will facilitate effective practices, promote compliance, and help ensure the well-being of those we serve.

Overview of Oklahoma's Regulatory Framework

In Oklahoma, the provision of services and support for individuals with developmental disabilities is governed by a rigorous regulatory framework. The Oklahoma Department of Human Services (DHS) plays a crucial role in overseeing Residential Habilitation Centers, ensuring that they adhere to state regulations, particularly those outlined in the Developmental Disabilities Services (OAC 340:100). This framework is designed to foster an environment of safety, dignity, and respect for all individuals receiving services.

Federal Medicare Conditions of Participation

In addition to state regulations, Residential Habilitation Centers must also comply with federal requirements outlined in the Medicare Conditions of Participation (42 CFR Part 484). These federal standards set forth essential requirements for healthcare facilities, ensuring that individuals receive necessary services in a safe and effective manner. Adhering to both state and federal guidelines is fundamental to the operational integrity of [AGENCY NAME].

Purpose of this Policy Manual

The purpose of this Policy & Procedure Manual is to provide [AGENCY NAME] with a clear framework for operational practices, enhancing quality service delivery, and ensuring compliance. This manual encompasses 41 essential policies categorized into Administrative, Clinical, Documentation, Financial, Infection Control, Personnel, Quality, and Safety. By standardizing procedures and policies, we aim to improve the overall quality of care and support provided to the individuals we serve.

Instructions for Use and Customization

This manual is designed to be a living document that can be adapted to meet the specific needs of [AGENCY NAME]. Each policy is provided with placeholders where agency-specific information can be filled in. We encourage agencies to customize the policies to reflect their unique operations, staffing structures, and service delivery models. To adapt these policies effectively, simply replace the [AGENCY NAME] placeholder with your agency's name and adjust any additional details that better fit your organizational context.

Compliance Commitment

At [AGENCY NAME], we are committed to upholding the highest standards of compliance with both state and federal regulations. This commitment is reflected in our adherence to the policies outlined in this manual, which are designed to enhance the quality of care and ensure the safety and well-being of every

individual we serve. By fostering a culture of accountability and continuous improvement, we strive to make a meaningful difference in the lives of individuals with developmental disabilities.

Vision, Mission & Core Values

Our Vision

[AGENCY NAME] envisions a future where every individual with disabilities lives with dignity, independence, and the opportunity to thrive within an inclusive community.

Our Mission

[AGENCY NAME] is dedicated to providing comprehensive residential habilitation services that empower individuals to achieve their personal goals and enhance their quality of life through compassionate, tailored support.

Core Values

Respect: We honor the dignity, individuality, and rights of every person we serve.

Empowerment: We empower individuals to take an active role in their own lives and decision-making processes.

Integrity: We uphold the highest standards of honesty and transparency in all our interactions.

Compassion: We provide empathetic and sensitive care, recognizing the unique needs and experiences of each individual.

Collaboration: We believe in the strength of teamwork and actively engage families, communities, and professionals to enhance our services.

Continuous Improvement: We are committed to ongoing learning and adaptation to enhance the quality of our services and outcomes.

How to Use This Manual

Manual Organization

This Policy and Procedure Manual is organized into the following sections:

- **Administrative Policies:** 11 policies
- **Clinical & Patient Care Policies:** 9 policies
- **Documentation & Records Policies:** 3 policies
- **Financial & Billing Policies:** 2 policies
- **Infection Control Policies:** 4 policies
- **Personnel & Staffing Policies:** 5 policies
- **Quality Policies:** 2 policies
- **Safety & Emergency Policies:** 5 policies

Policy Structure

Each policy in this manual follows a standardized format:

- **Purpose:** Why the policy exists and its objectives
- **Policy Statement:** The agency's commitment and standards
- **Procedures:** Step-by-step actions with responsible parties and timing
- **Responsibilities:** Role-based duties for implementation
- **Documentation:** Required records, forms, and retention requirements
- **Monitoring:** Audit methods, frequency, and corrective actions
- **Training:** Initial and ongoing training requirements

Customization Instructions

Before implementing this manual, replace all placeholders with your agency's specific information:

- `[AGENCY NAME]` - Your agency's legal name
- `[Agency Address]` - Physical address
- `[Contact Number]` - Main phone number
- `[Email Address]` - Primary email
- `[Effective Date]` - Date manual becomes effective
- `[Administrator Name]` - Name of approving authority

Regulatory Compliance

This manual is designed to comply with:

- Oklahoma state regulations for Residential habilitation centers
- Federal Medicare Conditions of Participation (42 CFR Part 484)
- Applicable accreditation standards (ACHC, CHAP, Joint Commission)

Administrative Policies

Section Overview

The "Administrative Policies" section of the Oklahoma Residential Habilitation Centers Policy & Procedure Manual outlines vital guidelines designed to ensure regulatory compliance and operational integrity. Comprising eleven comprehensive policies, this section focuses on essential areas such as governance, health information privacy, licensure, quality assurance, resident funds management, and accessibility. These policies are integral for maintaining the standards required for licensing and accreditation, guaranteeing that our centers not only meet state and federal regulations but also provide a safe and supportive environment for residents. By adhering to these policies, Residential Habilitation Centers can enhance service quality, promote ethical practices, and foster trust with stakeholders and the community.

Governing Body and Administrative Oversight Policy

State: `OAC 340:100-3-2` **Federal:** `42 CFR 483.410`

Policy Statement

This policy exists to establish an organized governance structure and administrative oversight necessary for the effective management of Residential Habilitation Centers in Oklahoma. It seeks to ensure compliance with both state and federal regulations, ultimately enhancing the quality of care and safety for individuals with disabilities residing in these facilities. By adhering to this policy, [AGENCY NAME] demonstrates its commitment to client welfare, staff responsibility, and operational excellence. [AGENCY NAME] shall maintain a clear organizational structure for governance and administration, ensuring that leadership responsibilities, administrative authority, and oversight mechanisms meet regulatory obligations. This policy covers all aspects of management necessary to achieve compliance with OAC 340:100-3-2 and 42 CFR 483.410.

Procedures

- **Governing Body Oversight:**
  - The Administrator shall establish and maintain a governing body that meets regularly to oversee the operations of the facility. Meetings will occur monthly, and the Administrator will ensure that meeting minutes and attendance records are documented and preserved for review.
- **Staff Background Checks:**
  - The HR Manager shall conduct background checks for all staff members prior to hiring, and annually thereafter. A log of these background checks shall be maintained to ensure compliance with regulatory standards.
- **Staff Training:**
  - The Training Coordinator shall provide mandatory training for all staff, focusing on core values, safety protocols, and client rights. This training shall be required upon hire and annually thereafter, with completion records documented and readily available for review.
  - All new staff must complete a comprehensive training program covering foundational values, rights of residents, and emergency procedures.
  - Annual refresher training will be implemented to update staff on compliance and best practices. Staff competency will be assessed during performance reviews and through direct observation.
- **Operational Compliance:**
  - The Compliance Officer shall implement an annual review of operational compliance with state and federal regulations. Compliance review reports shall be documented and provided to the governing body.
  - Regular audits and observations of staff compliance and operational protocols will be conducted by the Quality Assurance Team on a quarterly basis. Findings from these audits will be reported to the governing body, and immediate corrective action will be taken to address any non-compliance issues identified.

- **Documentation and Record Retention:**
  - [AGENCY NAME] shall maintain essential records including staff training records, compliance review reports, incident reports, and client care plans. All records must be securely stored with restricted access granted only to authorized personnel.
  - Necessary forms will include the Oklahoma Form LIC-XXXX and background check consent forms. All documentation must be retained for a minimum of five years.
- **Ongoing Monitoring and Improvement:**
  - The Quality Assurance Team will conduct regular assessments to ensure adherence to established protocols and identify areas for improvement. Compliance monitoring findings must lead to actionable insights for enhancing care quality and operational efficiency.

References

- **OAC 340:100-3-2**: Oklahoma Administrative Code on Residential Habilitation Centers
- **42 CFR 483.410**: Federal Regulations for Intermediate Care Facilities

Health Information Privacy and HIPAA Compliance Policy

State: `OAC 340:100-3-39` **Federal:** `45 CFR 160, 45 CFR 164`

Policy Statement

The Health Information Privacy and HIPAA Compliance Policy of [AGENCY NAME] is designed to safeguard the confidentiality of health information as mandated by state and federal regulations. This policy explicitly aims to ensure compliance with the Health Insurance Portability and Accountability Act (HIPAA) and relevant Oklahoma laws. By implementing this policy, [AGENCY NAME] commits to protecting the privacy of individuals receiving care in residential habilitation centers, thus enhancing patient safety and the quality of care. Through this policy, [AGENCY NAME] establishes comprehensive procedures governing the use, access, and disclosure of protected health information (PHI). All operations within [AGENCY NAME], including staff access to PHI, shall adhere to this policy. The agency is dedicated to maintaining a culture of privacy, ensuring all personnel are trained in HIPAA regulations, and developing incident response procedures for breaches of PHI.

Procedures

- **Access Controls:** The IT Manager shall establish access controls to limit PHI access to authorized personnel only. Access control logs shall be maintained starting from employee onboarding and continue annually thereafter.
- **Risk Assessments:** The Compliance Officer shall conduct annual risk assessments to identify vulnerabilities related to PHI, particularly following any data breaches. These assessments shall be documented in risk assessment reports.
- **Training Sessions:** The HR Director shall implement mandatory training sessions on HIPAA compliance for all staff upon onboarding and every 12 months thereafter. Training attendance records shall be maintained to verify compliance with training requirements.
- **Incident Documentation:** Upon discovery of a PHI breach, the Privacy Officer must document the breach immediately and establish a notification process. This includes preparing incident reports and sending breach notification letters as required.
- **Policy Review:** The Policy Review Committee is responsible for the regular review and update of privacy policies and procedures. This process occurs annually or as needed, with revisions documented in the updated policy documents.
- **Monitoring Compliance:** The Compliance Officer shall conduct internal audits and compliance reviews quarterly, or more frequently if non-compliance is suspected. The results of these reviews will be reported to the Administrator and the Board of Directors for oversight. Any identified non-compliance shall prompt the development and implementation of corrective action plans.
- **Documentation of Records:** All patient health records containing PHI, access control logs, training records, and incident and breach reports must be maintained securely. Documentation shall be retained for a period of five years and stored in locked cabinets or encrypted electronic systems to ensure confidentiality.
- **Initial and Ongoing Training:** All employees shall undergo mandatory HIPAA and privacy training during onboarding, followed by annual refresher training on HIPAA compliance and patient rights. Competency will be verified through quizzes and practical application evaluations, and attendance records will be maintained for all training sessions.

References

- **45 CFR 160**: HIPAA Privacy Regulations (link)
- **OAC 340:100-3-39**: Oklahoma Health Information Privacy Regulations (link)

Licensure, Certification, and Contractual Compliance Policy

State: `OAC 340:100-3-2, OAC 340:100-3-39, OAC Chapter 680`

Federal: `42 CFR 483.410`

Policy Statement

The Licensure, Certification, and Contractual Compliance Policy is established to ensure that the Residential Habilitation Centers in Oklahoma are compliant with all pertinent state and federal regulations governing the care and services provided to individuals with intellectual and developmental disabilities. This policy is designed to uphold the safety, well-being, and rights of residents, while simultaneously promoting high standards of quality care within the facility. By committing to this policy, [AGENCY NAME] fosters a structured environment that addresses the unique needs of our patients, ensuring that all operations align with regulatory expectations. To effectuate this commitment, [AGENCY NAME] shall maintain all required state licenses and federal certifications necessary to operate as a Residential Habilitation Center in Oklahoma. It is the responsibility of every employee to acknowledge and uphold these standards. Compliance with OAC 340:100-3-2, OAC 340:100-3-39, and 42 CFR 483.410 is mandatory, with key obligations involving timely management of licensure applications, comprehensive staff training in compliance areas, and consistent monitoring of adherence to established protocols.

Procedures

- The Administrator or Director shall complete the application for licensure and submit it to the Department at least 30 days prior to the license expiration. Documentation, such as the licensure application form, must be retained for a minimum of five years. - Upon hiring and annually thereafter, the Clinical Manager or Director of Nursing will conduct training related to compliance requirements. Training attendance records will be maintained in personnel files to document completion. - Supervisors are responsible for posting required compliance documents in a visible location within the facility immediately upon renewal of licenses or any changes. Evidence of these posted documents must be documented and accessible to relevant staff. - The Administrator or Director will perform regular internal audits for compliance on a quarterly basis. Audit reports will be generated and submitted to management to ensure ongoing monitoring of compliance with licensure and certification standards. - All incidents and compliance checks must be documented by Direct Care Staff as they occur. This includes incident reports and compliance check documentation, which must also be kept securely for five years. - Initial training will encompass compliance policies, incident reporting, and resident rights during employee orientation. Following the initial training, all staff must participate in annual refresher courses on compliance and client rights to ensure ongoing education. - Competency verification assessments will be conducted annually to validate that staff members fully understand compliance requirements. The results of these assessments must be recorded and kept in each employee's personnel file. - The Administrator or Director shall oversee all compliance monitoring activities, including internal audits and state inspections. The results from these audits are to be reported to agency management and discussed during regular staff meetings to foster awareness and accountability. - Should compliance issues arise during audits or inspections, corrective action plans must be implemented promptly. This will ensure that any identified deficiencies are addressed and that staff are informed of changes to compliance protocols. By adhering to these procedures, [AGENCY NAME] upholds its commitment to providing safe, high-quality care while ensuring compliance with all necessary regulatory standards.

References

- **OAC 340:100-3-2**: Oklahoma Administrative Code for Licensure (link)
- **42 CFR 483.410**: Federal Regulation for Intermediate Care Facilities (link)

Quality Assurance and Performance Improvement (QAPI) Policy

State: `OAC 340:100-3-27` **Federal:** `42 CFR 483.475`

Policy Statement

The Quality Assurance and Performance Improvement (QAPI) Policy of [AGENCY NAME] is established to ensure adherence to state and federal regulations dedicated to providing quality care in Residential Habilitation Centers. This policy aims to foster an environment of continuous improvement in services rendered to individuals with developmental disabilities, thereby enhancing their safety, well-being, and overall quality of life. Moreover, it serves to provide a solid framework for accountability among staff, reinforcing our commitment to excellence in care delivery. [AGENCY NAME] shall ensure that all services are aligned with the established guidelines while prioritizing the rights and well-being of the residents. This policy encompasses all operational aspects of care delivery, establishes criteria for performance measurement, and mandates continuous staff engagement in quality improvement practices, thereby reinforcing our commitment to superior care standards.

Procedures

- Develop a comprehensive QAPI plan that outlines specific goals and performance indicators.
  - **Responsible:** QAPI Coordinator
  - **Timing:** Annually at the start of the calendar year
  - **Documentation:** Written QAPI plan
- Conduct regular data collection on performance indicators and satisfaction metrics.
  - **Responsible:** Quality Improvement Committee
  - **Timing:** Quarterly
  - **Documentation:** Data collection reports
- Implement high-level oversight of the QAPI program.
  - **Responsible:** Administrator
  - **Action:** Ensure regulatory compliance and allocate resources for QAPI initiatives.
- Oversee clinical quality metrics and implement improvement strategies.
  - **Responsible:** Clinical Manager
  - **Action:** Monitor staff adherence to QAPI training and initiatives.
- Implement QAPI processes on a day-to-day basis.
  - **Responsible:** Supervisors
  - **Action:** Train direct care staff on their roles in QAPI.
- All staff members shall participate in initial QAPI foundational training within 30 days of hire.
- All staff must complete annual refresher courses on QAPI principles and their implementation.
- Direct care staff shall document care delivery appropriately and report any incidents that could impact the quality of care.

- Conduct regular audits and performance reviews to monitor compliance.
  - **Responsible:** Quality Improvement Coordinator
  - **Frequency:** Monthly
  - **Reporting:** Report results to the Administrative team and share findings with residents and families randomly.
  - **Corrective Action:** Establish a corrective action plan based on audit findings within 30 days.
- Maintain thorough documentation to support the QAPI program, including:
  - QAPI plan
  - Performance Improvement Project (PIP) documentation
  - Incident reports
  - Training records
- Use the following forms as part of QAPI documentation:
  - Oklahoma Form LIC-XXXX
  - Incident Reporting Form
- Maintain an appropriate retention period for all records related to QAPI activities for 5 years.
- Ensure that all QAPI records are stored confidentially, either in secure electronic or physical formats with restricted access.
- Conduct assessments of staff competency through direct observation and competency checklists annually to verify training effectiveness.
- Keep records of all training sessions and participant completion to maintain compliance with QAPI training requirements.

References

- **OAC 340:100-3-27**: Quality Assurance and Performance Improvement Policy Regulation (link)
- **42 CFR 483.475**: Federal Regulations on QAPI (link)

HCBS Settings Compliance Policy

State: `OAC 317:40` **Federal:** `42 CFR 441.301`

Policy Statement

This HCBS Settings Compliance Policy is instituted to ensure adherence to state and federal regulations pertaining to Home and Community-Based Services (HCBS) for individuals with disabilities, particularly as it applies to Oklahoma Residential Habilitation Centers. The aim of this policy is to uphold client dignity, autonomy, and protection while delivering safe and supportive services. [AGENCY NAME] is committed to providing services that recognize and affirm the rights of residents, meet regulatory standards, and foster inclusive community participation. This policy must be observed by all staff and operational activities within [AGENCY NAME]. It mandates compliance with the Oklahoma Administrative Code (OAC) 340:100 and 42 CFR 441.301, ensuring that client welfare and safety are prioritized in all activities. The agency pledges to uphold these standards through comprehensive training, diligent monitoring, and continuous improvement practices.

Procedures

- **Staff Background Checks:** The HR Manager shall ensure that all staff have completed the required background checks prior to their employment start date. This documentation must include confirmed records of background checks.
- **Core Training for Direct Care Staff:** The Training Coordinator will implement and document the completion of the Oklahoma DDS core training for all direct care staff within 30 days of their hire. Records of training completion certificates are to be maintained.
- **Client Rights Education:** The Training Coordinator is responsible for conducting annual client rights education sessions for staff, as well as additional sessions as needed. Attendance sheets and training materials must be documented.
- **Individualized Program Plan (IPP) Development:** Each Case Manager shall develop and review an Individualized Program Plan (IPP) for every client within 30 days of admission, with quarterly reviews thereafter. Client IPP documents must be maintained meticulously.
- **Monthly Audits:** The Quality Assurance Manager will conduct audits monthly to monitor compliance with documentation and client rights postings. Audit reports must be prepared to reflect findings.
- **Overall Governance and Compliance Oversight:** The Administrator shall oversee all governance and ensure regulatory compliance, approving policies and procedures, and reviewing compliance reports regularly.
- **Clinical Supervision:** The Clinical Manager is tasked with supervising clinical staff to ensure quality of care while monitoring client safety and adherence to care plans.
- **Daily Implementation and Support:** Supervisors will oversee the daily implementation of HCBS settings compliance, providing necessary training and support to direct care staff while ensuring documentation accuracy.
- **Direct Care Responsibilities:** Direct care staff must provide assistance to clients, documenting all interactions and client progress, and reporting incidents or concerns following established procedures.

- **Corrective Action Plans:** If deficiencies are identified during audits, a corrective action plan must be implemented by the Quality Assurance Manager within 30 days of such findings.
- **Training Documentation:** All staff training records must be maintained, documenting initial training on Oklahoma DDS core competencies and ongoing annual refresher training on client rights and HCBS compliance.
- **Competency Verification:** Ongoing competency of staff will be verified through direct observation and annual employee evaluations, ensuring that compliance and service quality standards are consistently met.

References

- **OAC 340:100**: Oklahoma Administrative Code (link)
- **42 CFR 441.301**: HCBS Setting Rule (link)

Resident Funds and Personal Property Management Policy

State: `OAC 340:100-3-8.1` **Federal:** `42 CFR 483.420(a)(5)`

Policy Statement

The purpose of this policy is to establish a comprehensive framework for the management of resident funds and personal property within Oklahoma Residential Habilitation Centers. This policy ensures compliance with OAC 340:100-3-8.1 and 42 CFR 483.420(a)(5). It aims to safeguard residents' financial interests by outlining clear procedures for the handling, tracking, and reporting of resident funds and property. By doing so, this policy is designed to protect residents from financial abuse while promoting transparency and accountability in financial management practices. [AGENCY NAME] is committed to ensuring that all staff are well-trained and thoroughly aware of these protocols. The agency will rigorously apply these procedures to protect the rights and welfare of residents. Additionally, necessary procedural safeguards will be implemented to adhere to regulatory requirements, enhancing the overall quality of care provided.

Procedures

- **Initial Assessment:**
  - The Social Worker shall conduct an initial assessment of each resident's financial status and their preference regarding funds management within 5 days of admission.
  - Documentation required: Initial Financial Assessment Form must be completed and stored securely.
- **Account Establishment:**
  - The Financial Administrator shall establish a personal funds account for each resident, ensuring that funds are managed securely before the end of the first week after admission.
  - Documentation required: Personal Funds Account Setup Form must be completed and maintained.
- **Monthly Statements:**
  - The Financial Administrator shall provide a monthly statement of account to residents or their legal representatives.
  - Documentation required: Monthly Account Statement must be issued monthly and filed appropriately.
- **Annual Audit:**
  - The Internal Auditor shall perform an annual audit of all resident funds accounts to ensure compliance with policies and protocols.
  - Documentation required: Annual Audit Report must be generated and reviewed with the Administrator, with findings incorporated into staff training as necessary.
  - Regular audits and internal reviews are to be conducted quarterly, with findings reported to the Administrator and used for ongoing training initiatives. Immediate rectification of any non-compliance issues identified during these audits is required.

- **Reporting Misuse:**
  - The Administrator shall report any suspected misuse of resident funds to the appropriate authorities as soon as suspicion arises.
  - Documentation required: Incident Report Form must be completed immediately following any incident of suspected misuse.
- **Staff Training:**
  - All staff must complete initial training on resident funds management, which includes appropriate handling of resident funds and understanding legal implications.
  - Ongoing education shall consist of annual refresher training on regulatory updates and compliance standards.
  - Competency verification shall be conducted through a checklist and knowledge tests to ensure staff are fully equipped to manage resident funds appropriately. Documentation of training attendance and completion records must be maintained securely for a period of 5 years.
- **Record Management:**
  - All records, including financial assessment forms, account opening documents, monthly financial statements, incident reports, and audit reports, must be stored in a secured, confidential electronic system with limited access.

References

- **OAC 340:100-3-8.1**: Oklahoma Administrative Code on Resident Funds Management (link)
- **42 CFR 483.420(a)(5)**: Federal Regulation for Resident Funds Protection (link)

Americans with Disabilities Act (ADA) Accessibility Policy

State: `Oklahoma Technical Assistance Document Section 508`

Federal: `ADA Title III`

Policy Statement

The purpose of the Americans with Disabilities Act (ADA) Accessibility Policy is to ensure compliance with the ADA and to foster an environment that is accessible and inclusive for all individuals with disabilities. This policy delineates the responsibilities of [AGENCY NAME] in providing reasonable accommodations and guaranteeing both physical and programmatic accessibility. By adhering to the regulations set forth by the ADA and relevant state laws, [AGENCY NAME] aims to protect the rights of residents, staff, and visitors, ensuring full participation in services provided at the residential habilitation center. It is the policy of [AGENCY NAME] to uphold the principles of accessibility by ensuring that all facilities are equipped to accommodate individuals with disabilities without discrimination. This policy encompasses all residents, staff, and visitors, reaffirming our commitment to compliance with applicable federal and state regulations regarding accessibility.

Procedures

- **Conduct an ADA accessibility assessment**
  - Action: Assess the facility to identify areas requiring improvement.
  - Responsible: Facility Manager
  - Timing: Annually or as needed following any major renovations.
  - Documentation: Maintain an ADA Accessibility Assessment Report.
- **Implement modifications**
  - Action: Make necessary modifications based on findings from the accessibility assessment.
  - Responsible: Maintenance Supervisor
  - Timing: Within 90 days of completion of the assessment.
  - Documentation: Keep a Record of Modifications Made.
- **Distribute reasonable accommodation request form**
  - Action: Create and provide a form for residents to request reasonable accommodations.
  - Responsible: Administrative Assistant
  - Timing: Upon implementation of this policy.
  - Documentation: Utilize the Oklahoma Reasonable Accommodation Request Form.
- **Staff training on ADA compliance**
  - Action: Provide training regarding ADA compliance and resident rights.
  - Responsible: Training Coordinator
  - Timing: Annually, with initial training upon hire.
  - Documentation: Maintain Training Attendance Records and conduct competency assessments post-training.

- **Establish complaint process**
  - Action: Develop a procedure for residents to report accessibility issues.
  - Responsible: Director
  - Timing: Immediately upon implementation of this policy.
  - Documentation: Keep Complaint Procedure Documentation accessible for review.
- **Monitor compliance**
  - Action: Conduct annual audits to assess adherence to accessibility policies.
  - Frequency: Annually.
  - Responsible: Compliance Officer
  - Reporting: Results shared with the agency director and staff.
  - Corrective Action: Perform periodic reviews to determine necessary actions for any identified deficiencies.

Records must be securely stored to ensure the confidentiality of residents' information, with a retention period of 5 years for documentation including the ADA Accessibility Assessment Report, Record of Modifications Made, Reasonable Accommodation Request Form, Training Attendance Records, and Complaint Procedure Documentation. These procedures ensure that [AGENCY NAME] remains compliant with ADA regulations while providing a supportive environment for all individuals.

References

- **42 CFR 483.400-483.480**: Conditions of Participation for Intermediate Care Facilities (link)
- **ADA Title III**: Americans with Disabilities Act Title III Regulations (link)

Accreditation and Organizational Ethics Policy

State: `ACHC/CHAP Standards` **Federal:** `N/A`

Policy Statement

This Accreditation and Organizational Ethics Policy is established to ensure compliance with the Oklahoma Administrative Code 340:100 and relevant federal regulations governing Residential Habilitation Centers. The purpose of this policy is to foster a culture centered on quality care, safety, ethics, and organizational accountability within [AGENCY NAME]. It aims to protect the rights of residents and create a supportive environment for staff to deliver optimal services. [AGENCY NAME] is committed to upholding principles of transparency, non-retaliation, and consistent ethical behavior across all levels of operation. This policy applies to all staff and operations within [AGENCY NAME], ensuring that all practices comply with the established state and federal standards. It emphasizes the importance of preserving the dignity and rights of residents while providing comprehensive training and support for staff.

Procedures

- **Ensure Staff Training:** All staff shall complete the required Oklahoma DDS core training before they receive direct assignments. The Training Coordinator is responsible for overseeing this training, which must be documented through training completion records.
- **Review Individual Program Plans (IPPs):** The Clinical Manager shall conduct quarterly reviews of individual program plans (IPPs) to ensure they comply with residents' needs and rights. Documentation, including IPPs and review notes, must be maintained accurately.
- **Residents' Rights and Complaint Procedures:** The Facility Administrator shall post and maintain visibility of residents' rights and complaint procedures within the facility. This task is ongoing, and verification of postings must be documented.
- **Monitoring Compliance:** The Quality Assurance Manager shall conduct routine audits and compliance checks monthly. The results of these audits will be reported to the Administrator and the Department of Public Health. Should any deficiencies be identified, corrective actions must be initiated within 30 days.
- **Incident Reporting and Documentation:** Direct care staff must report incidents according to established protocols. Incident reports should be documented using the designated incident report form. All records, including training records, incident reports, and IPP documentation, shall be retained securely for a period of five years and kept accessible only to authorized personnel.
- **Ongoing Training:** All new staff must complete the Oklahoma DDS core training before engaging with residents. Additionally, annual refresher training on accreditation standards and ethical practices shall be required for all staff. Competency assessments will be conducted annually to ensure staff proficiency, with records of training attendance and assessments maintained.
- **Policy Adherence and Oversight:** The Administrator shall oversee the implementation of this policy, ensuring regulatory compliance and managing audits and inspections. Supervisors will assist by training staff on policy adherence and reviewing documentation for compliance, while also handling resident complaints.

References

- **OAC 340:100**: Oklahoma Administrative Code for Developmental Disabilities Services (link)
- **42 CFR 483.400-483.480**: Conditions of Participation for ICF/IID (link)

Survey and Regulatory Oversight Compliance Policy

State: `OAC Chapter 680:3-9` **Federal:** `N/A`

Policy Statement

The Survey and Regulatory Oversight Compliance Policy is designed to establish procedures that will guarantee compliance with state regulations under OAC Chapter 680:3-9 as well as applicable federal guidelines. This policy is vital for maintaining a high standard of care within Oklahoma's Residential Habilitation Centers. By promoting safety, quality service delivery, and the protection of individual rights, this policy serves the interests of clients, staff, and [AGENCY NAME] as a whole. It provides a comprehensive framework that supports operational effectiveness, risk management, and legal compliance. [AGENCY NAME] is committed to ensuring that all operations adhere to regulatory requirements while maintaining the highest quality of care. This policy reflects our dedication to upholding the integrity of services delivered to clients. Key obligations include strict compliance with both state and federal regulations, adherence to operational procedures, and the timely reporting and remediation of any compliance deficiencies identified.

Procedures

- **Conduct a Comprehensive Compliance Review:**
  - The Compliance Officer shall conduct a thorough review of all operational procedures to ensure they align with OAC Chapter 680:3-9.
  - This review will take place annually and will be documented in a Compliance Review Report.
- **Prepare for External Surveys:**
  - The Administrator is responsible for preparing for external surveys by documenting readiness.
  - Preparation shall include the collection of compliance evidence and staff preparedness checks, occurring at least 30 days before each scheduled survey.
  - Completion of this step will be confirmed through a Survey Preparedness Checklist.
- **Respond to Deficiencies:**
  - The Administrator must develop and implement a corrective action plan for any deficiencies identified in survey results.
  - This plan shall be created within 10 days of receiving survey results and maintained as a Corrective Action Plan.
- **Post Compliance Documentation:**
  - The Compliance Officer shall ensure that survey results and compliance documentation are posted in a prominent area within the facility immediately upon receipt of the survey results.
  - Documentation of the posting shall be maintained using a Survey Results Posting Confirmation.
- **Conduct Internal Audits:**
  - The Internal Auditor is tasked with conducting internal audits quarterly to verify compliance with implemented corrective action plans.

- Results of these audits will be documented in an Internal Audit Report, which will be reported to the Administrator and the Board.
- Corrective action for any instances of non-compliance must be implemented within 14 days of audit findings.

- **Training Requirements:**
  - All new staff shall complete training on compliance procedures within their first week of employment.
  - Annual refresher training on compliance and regulatory changes is mandatory for all staff to ensure ongoing education and awareness.
  - Competency assessments shall be conducted annually to verify staff understanding and adherence to compliance protocols.
  - Training records must be maintained securely and be available for review, in compliance with retention requirements of five years.

- **Responsibilities of Staff:**
  - The Administrator shall oversee compliance with all regulatory requirements, manage corrective action plans and compliance audits, and ensure staff are trained on compliance procedures.
  - The Clinical Manager is responsible for supervising the delivery of quality care, monitoring staff training and competency evaluations, and ensuring the implementation of individualized service plans.
  - Supervisors must ensure daily operations align with compliance standards, facilitate training and onboarding regarding state regulations, and review documentation compliance daily.
  - Direct Care Staff are required to report any incidents or compliance issues to their supervisors, participate in required training and competency assessments, and follow documented procedures for care delivery.

References

- **OAC 340:100**: Oklahoma Administrative Code
- **42 CFR 483.400-483.480**: Conditions of Participation for ICF/IID
- **CMS State Operations Manual Appendix J**: CMS Guidance for ICF/IID Facilities

Admission, Eligibility, and Discharge Policy

State: `OAC 340:100-5-3, OAC 340:100-3-2, OAC 317:30-5-122, OAC 317:35-9-45`

Federal: `42 CFR 483.440`

Policy Statement

[AGENCY NAME] is committed to ensuring compliance with all applicable state and federal regulations governing the admission, eligibility, and discharge processes for residents in Residential Habilitation Centers. This policy is established to provide clear guidelines that protect the rights of individuals receiving services, maintain the quality and safety of care, and ensure that all procedures implemented align with Oklahoma state regulations and federal mandates. The ultimate goal is to not only achieve compliance but also enhance service delivery to improve outcomes for residents and promote their overall well-being. This policy applies to all staff involved in the admission and discharge processes. To uphold this commitment, [AGENCY NAME] shall implement thorough procedures that ensure a seamless flow from the admission process through to discharge. Each responsible staff member is expected to adhere to these guidelines diligently to foster a supportive environment for all residents.

Procedures

- **Complete Admission Application:**
  - Action: The Intake Coordinator shall complete the application form for admission provided by [AGENCY NAME].
  - Timing: This shall be done immediately upon the request for admission.
  - Documentation: A completed application form shall be maintained as part of resident records.
- **Conduct Intake Assessment:**
  - Action: A qualified professional shall conduct an intake assessment to determine eligibility for services.
  - Timing: This assessment shall occur within 5 business days of application submission.
  - Documentation: An intake assessment report shall be documented and stored as part of the resident's record.
- **Review Assessment Findings:**
  - Action: The Admission Committee shall review the intake assessment findings.
  - Timing: This review will be conducted within 2 business days of assessment completion.
  - Documentation: Meeting minutes and a decision record shall be prepared to document the outcome.
- **Notify Admission Decision:**
  - Action: The Intake Coordinator shall notify the applicant and their family members of the admission decision.
  - Timing: Notification shall occur within 1 business day of the committee’s decision.
  - Documentation: An admission notification letter shall be sent and archived.

- **Develop Individualized Service Plan (ISP):**
  - Action: The Service Coordinator shall develop an ISP that addresses the specific needs of the resident.
  - Timing: The ISP must be completed within 30 days of admission.
  - Documentation: The completed ISP shall be recorded and made part of the resident's official documentation.
- **Initiate Discharge Planning:**
  - Action: The Service Coordinator shall initiate discharge planning at the time of admission and review it regularly.
  - Timing: This planning shall be an ongoing task from admission until the resident is discharged.
  - Documentation: All discharge planning documentation must be maintained and reviewed as needed.
- **Quality Monitoring:**
  - Action: The Compliance Officer shall conduct regular audits of documentation and procedures followed during the admission and discharge processes.
  - Frequency: Audits shall be performed quarterly to ensure compliance.
  - Reporting: Results shall be reported to the Administrator and utilized for staff training sessions.
  - Corrective Action: If deficiencies are identified, action plans must be developed, and follow-up assessments shall be conducted to ensure compliance.
- **Staff Training:**
  - Action: All staff shall receive an orientation on admission and discharge processes before interacting with residents.
  - Ongoing Education: Staff shall undergo annual refresher training on compliance and procedural updates.
  - Competency Verification: Staff performance reviews shall include documented assessments to verify competency in these areas.
  - Documentation: Training records must be maintained and updated regularly to ensure all staff are compliant with training requirements.

References

- **OAC 340:100-5-3**: Oklahoma Administrative Code for Residential Habilitation Centers
- **42 CFR 483.440**: Federal Conditions of Participation for Intermediate Care Facilities

Posting of Compliance and License Documentation Policy

State: `OAC 340:100-3-34, OAC Chapter 680:3-10, OAC Chapter 680:3-11`

Federal: `N/A`

Policy Statement

The purpose of the Posting of Compliance and License Documentation Policy is to ensure that all required compliance and licensing documentation is consistently and prominently displayed within the Residential Habilitation Centers. This policy is essential to meet the regulatory requirements established by the State of Oklahoma. By ensuring transparency and accessibility of vital information, we support the rights of residents, staff, and stakeholders, while maintaining safety and quality standards within the facility. In line with this commitment, [AGENCY NAME] shall implement and uphold a Posting of Compliance and License Documentation Policy that adheres to OAC 340:100-3-34 and related regulations. To fulfill this purpose, [AGENCY NAME] shall ensure that all necessary documents, including the current state license, residents' rights, and complaint procedures, are posted conspicuously for easy access by residents, staff, and visitors, promoting an informed and compliant environment.

Procedures

- **Verify Current License:** The Administrator shall verify that the current state license is valid and maintained on file immediately upon receipt of license renewal. Documentation of the current state license must be retained.
- **Post Residents' Rights:** The Clinical Manager shall post the residents' rights information in the common area within 24 hours of updating this information. Evidence of posting residents' rights must be documented and maintained.
- **Display Complaint Procedures:** The Administrator must ensure that the complaint procedures are clearly displayed in a prominent location. This shall be reviewed monthly, with documentation showing the posting date retained for compliance.
- **Quarterly Compliance Checks:** Supervisors shall conduct a quarterly check of all compliance postings for accuracy. A compliance check report must be documented to serve as a record of findings and actions taken.
- **Compliance Responsibilities:**
    - The Administrator is responsible for ensuring ongoing compliance with state and federal regulations. They will review and approve all postings annually and oversee the accuracy of all compliance documents.
    - The Clinical Manager shall monitor the implementation of posting procedures, train staff on correct posting practices, and report any discrepancies in postings to the Administrator.
    - Supervisors will conduct routine checks on posting compliance, document findings from compliance checks, and provide feedback and training to direct care staff to ensure ongoing adherence to this policy.
    - Direct Care Staff must ensure that all posted documents are accurate and up-to-date. They shall report any issues regarding document postings to supervisors and facilitate residents' understanding of their rights and procedures.

- **Documentation & Retention:** Records including the current state license, residents' rights document, complaint procedure document, and postings check report must be stored in a secure, accessible location in compliance with HIPAA standards for a period of five years.
- **Monitoring and Reporting:** Routine audits and checks of posted documents for compliance shall be performed quarterly, with an annual comprehensive review conducted by Supervisors and the Administrator. The results are to be documented and reported back to the Administrator, who will utilize the findings for further staff training. Immediate corrections to postings are required, and retraining will occur if necessary.
- **Training Requirements:**
    - All staff must undergo training on the Posting of Compliance and License Documentation Policy during their orientation process.
    - Annual refresher training will be provided to ensure all staff remain familiar with their responsibilities regarding compliance documentation.
    - Staff competency will be verified through performance evaluations focusing on their knowledge of compliance postings, and training records must be maintained for all staff to track their compliance training history.

Clinical & Patient Care Policies

Section Overview

The "Clinical & Patient Care Policies" section serves as a comprehensive framework for ensuring regulatory compliance within Oklahoma Residential Habilitation Centers. This collection of nine essential policies focuses on key areas such as nutrition, medication management, active treatment, individual program planning, and resident rights. By adhering to these standards, agencies can provide safe, effective, and patient-centered care, emphasizing the well-being of residents. These policies are crucial not only for maintaining licensing and accreditation but also for fostering an environment that promotes dignity, respect, and individualized support. Ultimately, they reflect our commitment to delivering high-quality services that align with best practices in the field.

Nutrition, Dietary, and Food Service Policy

State: `OAC 340:100-5-30, OAC 340:100-5-31` **Federal:** `42 CFR 483.480`

Policy Statement

The purpose of this policy is to establish comprehensive standards and procedures for Nutrition, Dietary, and Food Service in Oklahoma Residential Habilitation Centers and to ensure compliance with all applicable state and federal regulations. It is essential that each individual receives nutritionally adequate meals tailored to their specific health needs. [AGENCY NAME] is committed to the importance of individualized dietary assessments, menu planning, food safety, sanitation practices, and compliance monitoring to promote the health and well-being of residents. [AGENCY NAME] shall adhere strictly to all relevant state and federal regulations, ensuring not only the safety of residents but also respecting their individual dietary preferences and requirements. This policy outlines a systematic, thorough approach to maintaining high standards in dietary services, thereby enhancing the quality of care provided to residents.

Procedures

- **Individualized Dietary Assessments:**
  - Conduct individualized dietary assessments for each resident within 14 days of admission and at least annually thereafter to identify each resident's nutritional needs. Responsible: Dietary Manager
  - Ensure that dietary assessment forms are completed and signed, and these records are maintained for a retention period of 5 years. Documentation required.
- **Menu Development:**
  - Develop and maintain a menu that reflects the dietary assessments and adheres to nutritional guidelines. Responsible: Dietary Manager
  - Review and adjust the menu on a monthly basis as necessary, ensuring all menu documentation is filed monthly. Documentation required.
- **Food Safety Protocols:**
  - Implement food safety protocols during food preparation, cooking, and serving to prevent foodborne illness. Responsible: Food Service Staff
  - Complete food safety checklists daily to document adherence to safety protocols. Documentation required.
- **Sanitation Inspections:**
  - Conduct regular sanitation inspections of kitchen and dining areas weekly to ensure compliance with health standards. Responsible: Quality Assurance Manager
  - Document and file inspection reports to maintain a thorough record of compliance. Documentation required.
- **Compliance Monitoring:**
  - Monitor compliance with the menu and dietary service standards through audits and direct observations bi-monthly. Responsible: Area Supervisors

- Compile and submit audit reports to management, ensuring that any findings are documented. Documentation required.
- Develop a corrective action plan for any non-compliance identified during monitoring and ensure that corrective measures are implemented effectively. Responsible: Quality Assurance Manager

- **Staff Training Requirements:**
  - All new staff must complete initial training on dietary services, food safety, and nutrition upon hire. Documentation of training records is required.
  - Provide ongoing annual refresher training on food safety and dietary guidelines to maintain high competency levels. Documentation of training updates also required.
  - Assess staff competency through practical tests and observations during training sessions to ensure they meet required standards. Documentation for competency verification must be maintained.

References

- **OAC 340:100-5-30**: Nutritional Requirements and Dietary Services
- **OAC 340:100-5-31**: Food Services Requirements
- **42 CFR 483.480**: Requirements for Nutrition and Dietary Services

Medication Administration, Storage, and Management Policy

State: `OAC 340:100-5-52, OAC 340:100-5-32` **Federal:** `42 CFR 483.460`

Policy Statement

The purpose of this policy is to establish comprehensive protocols for the administration, storage, and management of medications within residential habilitation centers in Oklahoma. This policy is designed to ensure compliance with state and federal regulations, prioritize patient safety, and maintain high standards of quality care. By adhering to these guidelines, [AGENCY NAME] shall implement effective practices in medication management that comply with OAC 340:100-5-52, OAC 340:100-5-32, and 42 CFR 483.460. This commitment extends to all staff engaged in medication handling, administration, and oversight. To this end, [AGENCY NAME] shall create an environment focused on protecting and benefiting residents, staff, and the organization as a whole. Every caregiver must recognize their pivotal role in medication management to uphold these standards of practice and accountability.

Procedures

- **Identity Verification:** Direct Care Staff shall verify the identity of the client using two identifiers before administering any medication. This verification process must occur before each administration of medication, and staff are required to document client identity verification in the medication administration record.
- **Medication Administration:** Direct Care Staff shall administer medications as prescribed by the physician, following the established medication schedule. Proper documentation of the medication given, dosage, time, and any observations regarding the resident's reaction must be maintained in the client's health records.
- **Medication Storage:** The Clinical Manager shall ensure that all medications are stored securely in a locked cabinet or designated area to prevent unauthorized access. This measure must be performed daily, with a record of medication inventory and any discrepancies being maintained for monitoring purposes.
- **Medication Disposal:** The Clinical Manager is responsible for the disposal of expired or unused medications in accordance with state regulations. This process should be conducted monthly or as needed, with documentation using the approved disposal form to record all disposal transactions.
- **Error Reporting:** All Staff must report any medication errors immediately following internal reporting procedures. This includes completing a medication error report form and notifying the supervisor upon discovery of the error.
- **Client Monitoring:** Direct Care Staff shall continuously monitor clients for adverse effects and medication interactions after medication administration. Any observed side effects must be documented in the client's health record to ensure thorough tracking and response.
- **Training Requirements:** All staff members must complete DDS Core Training, which includes comprehensive education on medication administration protocols, prior to their assignment. Ongoing annual refresher training regarding medication procedures and updates shall be mandated to maintain high competency levels. Competency will be verified through direct observations and assessments during the probationary period, with complete training records for all staff involved in medication management being maintained to ensure accountability.

- **Compliance Monitoring:** The Clinical Manager shall conduct regular audits of medication administration processes and records at least quarterly. Audit findings will be summarized in a report to the agency director. Should compliance issues be identified during the audits, corrective actions must be implemented, accompanied by additional training as needed to prevent recurrence of such issues.

References

- **OAC 340:100-5-52**: Oklahoma Administrative Code on Medication Administration
- **OAC 340:100-5-32**: Oklahoma Administrative Code on Medication Management
- **42 CFR 483.460**: Federal Regulations on Medication Administration for ICF/IID (link)

Active Treatment and Habilitation Policy

State: `OAC 340:100, OAC 317:40` **Federal:** `42 CFR 483.440`

Policy Statement

The purpose of the Active Treatment and Habilitation Policy is to define the principles and operational steps necessary for providing continuous active treatment, aimed at fostering skill acquisition, choice, and independence among the individuals served at Oklahoma Residential Habilitation Centers. This policy is critical for ensuring compliance with Oklahoma state regulations outlined in OAC 340:100, as well as applicable federal standards under 42 CFR 483.440. By adhering to this policy, [AGENCY NAME] is committed to enhancing the quality of life for all residents by respect and address of their individual needs, rights, and preferences. [AGENCY NAME] shall deliver active treatment services that are tailored to each individual. This will include conducting appropriate assessments, developing individualized program plans (IPPs), and maintaining continuous support to promote skill development that leads to greater independence and community integration. All staff members involved in resident care and rehabilitation are required to follow this policy, reaffirming our commitment to the welfare and development of those in our centers.

Procedures

- Conduct an initial assessment of each resident to identify unique needs and strengths. This responsibility lies with the Clinical Manager and must be completed within 72 hours of admission. The Initial Assessment Report shall be documented and maintained as part of the resident's record.
- Develop an Individualized Program Plan (IPP) based on the assessment findings. Direct Care Staff are responsible for this step, which should be completed within 7 days of the assessment's completion. The IPP shall be documented using the Oklahoma Form 340:100-001 - Individualized Program Plan Template.
- Implement the IPP through structured activities aimed at skill development. Direct Care Staff will execute these activities daily according to the IPP schedule, while documenting progress in daily logs. Document results using the Oklahoma Form 340:100-002 - Daily Activity Log.
- Review and update the IPP quarterly to reflect any changes in resident needs. The Clinical Manager is responsible for conducting these reviews, maintaining documentation in the IPP Review Report. This process ensures that each resident's care plan remains dynamic and responsive to their progress.
- Ensure overall compliance with state and federal regulations as the responsibility of the Administrator, who will also approve all individualized program plans. This oversight includes monitoring the implementation of active treatment services and ensuring necessary staff training on these principles.
- Supervise direct care staff to ensure adherence to individualized plans. Supervisors shall conduct regular assessments of staff performance and compliance, coordinating staff schedules to allow for appropriate treatment activities.
- Provide staff training on active treatment principles and documentation requirements annually. The Administrator will oversee the training sessions and maintain a Training Attendance Record to document participation and competencies achieved by the staff.

- Conduct regular audits of documentation and adherence to individualized program plans to monitor compliance. This responsibility falls under the Quality Assurance Officer, with audits happening quarterly. Results of these audits should be reported to both the Administrator and Clinical Manager to implement corrective measures for any identified deficiencies.
- During staff orientation, offer comprehensive training on active treatment principles. Additionally, conduct annual refresher training on documentation and resident rights as ongoing education. Competency verification will occur through periodic evaluations and direct observation, ensuring that staff remain proficient in their roles.
- Maintain all records, including Initial Assessment Reports, Individualized Program Plans, Daily Activity Logs, IPP Review Reports, and Training Attendance Records, in a secure and confidential manner for a period of five years. Access to these records shall be limited to authorized personnel only.

References

- **OAC 340:100-5**: Oklahoma Administrative Code for Developmental Disabilities Services (link)
- **42 CFR 483.440**: Federal Regulation on Active Treatment for ICF/IID (link)

Restraint, Seclusion, and Behavior Intervention Policy

State: `OAC 340:100-5-57` **Federal:** `42 CFR 483.430(e)`

Policy Statement

The Restraint, Seclusion, and Behavior Intervention Policy is established to ensure that [AGENCY NAME] complies with all state and federal regulations governing the use of restraint, seclusion, and behavior intervention practices within Residential Habilitation Centers. This policy is imperative for safeguarding the rights and welfare of our clients while equipping our staff with the necessary knowledge and procedures to maintain a safe care environment. Through regular oversight, accountability, and a commitment to continuous improvement, we aim to prioritize the dignity and individualized care of each individual served. [AGENCY NAME] commits to implementing comprehensive guidelines concerning the use of restraint and seclusion, ensuring these interventions are applied only in absolute necessity and are thoroughly documented. The measures prescribed in this policy apply to all staff members within the Residential Habilitation Centers, reinforcing our dedication to the highest standards of care and ethical treatment of individuals.

Procedures

- **Behavioral Assessment:**
  - Conduct a behavioral assessment for each resident within 30 days of their admission to the center.
  - *Responsible Party:* Behavioral Specialist
  - *Documentation:* Behavioral assessment report must be maintained securely.
- **Staff Training:**
  - All staff shall undergo training on restraint, seclusion, and behavior intervention-related policies and procedures during the onboarding process.
  - Annual refresher training shall be provided to ensure compliance with updated practices.
  - *Responsible Party:* Training Coordinator
  - *Documentation:* Training attendance records must be filed and maintained.
- **Incident Documentation:**
  - Document any use of restraint or seclusion in the resident's medical record immediately following the incident.
  - *Responsible Party:* Direct Care Staff
  - *Documentation:* An incident report must be prepared and filed promptly.
- **Monthly Review:**
  - Review all incidents of restraint or seclusion monthly to ensure compliance with the policy and identify areas for improvement.
  - *Responsible Party:* Clinical Manager
  - *Documentation:* Monthly compliance review reports shall be generated and retained.
- **Guardian Notification:**
  - Notify guardians of any use of restraint or seclusion within 24 hours of the incident.

- *Responsible Party:* Direct Care Staff
- *Documentation:* A notification log must be maintained to document these communications.

- **Monitoring Compliance:**
  - Conduct monthly reviews of restraint and seclusion incidents to monitor compliance with policy and procedures.
  - *Responsible Party:* Clinical Manager
  - The results of these reviews shall be reported to the Administrator for further oversight.
  - Implement immediate retraining for any staff involved in non-compliant incidents to enhance adherence to policy.

- **Record Retention:**
  - Maintain the following records for a retention period of five years:
    - Behavioral assessment reports
    - Training attendance records
    - Incident reports
    - Monthly compliance review reports
    - Notification logs
  - Ensure all records are stored securely to maintain confidentiality and access control.

References

- **42 CFR 483.430**: Conditions of Participation for Intermediate Care Facilities for Individuals with Intellectual Disabilities (link)
- **OAC 340:100-5-57**: Oklahoma Administrative Code (link)

Individual Program Plan (IPP) and Person-Centered Planning Policy

State: `OAC 340:100-5, OAC 317:40` **Federal:** `42 CFR 483.440`

Policy Statement

[AGENCY NAME] shall ensure that each individual receiving services has a comprehensive Individual Program Plan (IPP) that reflects their unique preferences, strengths, and needs. This policy is established to comply with both state and federal regulations while promoting the well-being, safety, and rights of all residents. By prioritizing person-centered approaches, we commit to enhancing the quality of care and support provided, ensuring individuals are active participants in their own planning processes and that their rights are upheld throughout their stay in our facilities. It is the policy of [AGENCY NAME] to adopt an individualized approach to care through the establishment and implementation of Individual Program Plans. Every individual's IPP shall be developed with full involvement from the individual and their support network, ensuring that their unique voice and choices are reflected in their care.

Procedures

- **Assessment Phase:**
  - The Direct Care Staff shall conduct a comprehensive assessment of the individual's needs, strengths, and preferences within 30 days of admission. This assessment will be documented in an assessment report filed in the individual's record.
- **Development of Individual Program Plan (IPP):**
  - The Clinical Manager shall lead the development of the IPP in collaboration with the individual and their support network within 45 days of admission. The completed IPP must be signed and dated by all participants to ensure mutual agreement and understanding.
- **Implementation:**
  - Once the IPP is developed, Supervisors must implement the plan immediately, ensuring all staff are trained and aware of their specific roles in delivering the outlined services.
  - Documentation of the implemented IPP and training sessions shall be maintained through staff training logs and IPP implementation tracking to ensure ongoing adherence to the plan.
- **Monitoring and Quality Assurance:**
  - The Clinical Manager shall review and update the IPP at least annually or more frequently if significant changes occur in the individual's progress. Updated IPPs must be documented and signed accordingly.
  - The Administrator shall conduct routine audits of IPP implementation and compliance reviews quarterly. Audit findings shall be reported to the agency board, and any necessary corrective measures must be communicated to staff to ensure continuous improvement.
  - Non-compliance identified during audits will necessitate additional training or the creation of corrective plans based on the audit results.

- **Training Requirements:**
  - All new staff shall undergo training on person-centered planning and IPP processes during their orientation. This initial training is essential for fostering a thorough understanding of the policy and procedure.
  - Staff must participate in annual refresher training covering regulatory updates and best practices, reinforcing their knowledge and skills regarding person-centered care.
  - Competency verification shall be conducted to ensure staff demonstrate understanding through competency assessments following training sessions. Documentation of all training records must be maintained for each staff member to ensure compliance and accountability.

References

- **42 CFR 483.440**: Conditions for Participation for ICFs/IID (link)
- **OAC 340:100-5**: Oklahoma Administrative Code for Developmental Disabilities Services

Consent for Care and Treatment Policy

State: `OAC 340:100-5-22.1` **Federal:** `42 CFR 483.420(a)(2)-(3)`

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Policy Statement

This Consent for Care and Treatment Policy is established to ensure compliance with Oklahoma state regulations and federal laws regarding informed consent in the care and treatment of residents in Residential Habilitation Centers. The policy outlines the necessary steps for obtaining and documenting informed consent that not only respects resident autonomy but also safeguards their rights. This policy is designed to benefit residents by ensuring they have the crucial information needed to make informed choices regarding their care and treatment. [AGENCY NAME] is committed to providing all care and treatment to residents only with their informed consent, thereby upholding their rights and choices throughout the care process. This policy applies to all staff and operations at [AGENCY NAME] involved in resident care and treatment, emphasizing the need for compliance with regulations, thorough documentation of consent, and continuous staff training to maintain these practices.

Procedures

- **Initiation of Consent Process:** Prior to the initiation of any treatment, Direct Care Staff shall provide the resident or their legal representative with comprehensive information about the proposed treatments. This information must include potential benefits, risks, and available alternatives. Documentation of this process shall be ensured by obtaining a signed and dated Informed Consent Form.
- **Documentation of Consent:** Immediately following the consent discussion, the Nurse shall document the consent process in the resident's medical record. This documentation must capture details of the discussion, decisions made, and a copy of the signed Informed Consent Form must be included to substantiate that consent was obtained.
- **Updating Consent Documentation:** The Clinical Manager shall review and update consent documentation whenever changes in treatment plans or procedures arise. It is imperative to maintain an accurate and updated Informed Consent Form as part of the ongoing resident record.
- **Monitoring Compliance:** To ensure adherence to consent protocols, the Quality Assurance Officer shall conduct regular audits of consent processes and documentation on a quarterly basis. The results of these audits will be reported to the Administrator and discussed in staff meetings, with immediate corrective action plans implemented for any identified deficiencies.
- **Training Requirements:** All staff are required to complete initial training on informed consent processes during their orientation. Ongoing annual refresher training on consent laws and policies will also be mandatory for all employees. To ensure competency, biannual assessments will be conducted through staff evaluations, with documentation of all training records maintained within personnel files.
- **Retention of Documentation:** As part of the documentation requirements, the Informed Consent Forms and all associated resident medical records reflecting consent discussions shall be retained for a minimum of five years. All records must be confidentially stored in a secure electronic health record system to maintain the privacy and security of resident information.

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### References

- **OAC 340:100-5-22.1**: Oklahoma Administrative Code on Consent for Care (link)
- **42 CFR 483.420(a)(2)-(3)**: Federal Regulations regarding Informed Consent (link)
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Resident Rights and Protections Policy

State: `OAC 340:100-5-22.1, OAC Chapter 680` **Federal:** `42 CFR 483.420`

Policy Statement

The Resident Rights and Protections Policy exists to ensure compliance with state and federal regulations regarding the rights of residents in Residential Habilitation Centers. This policy is meticulously designed to foster an environment that respects and protects the dignity, privacy, and rights of all residents. By doing so, it aims to prevent abuse, neglect, and discrimination, while promoting a safe and supportive living environment. Establishing clear guidelines for the treatment of residents not only safeguards their rights but also enhances their overall quality of life. [AGENCY NAME] shall maintain a firm commitment to uphold resident rights in accordance with OAC 340:100-5-22.1 and 42 CFR 483.420. All staff members must familiarize themselves with these rights and diligently implement them in daily operations. Residents are ensured freedom from abuse and neglect, the right to express grievances, and the right to participate actively in decisions affecting their care and treatment. The management at [AGENCY NAME] shall uphold the responsibility of monitoring compliance with these regulations, whilst ensuring that necessary training is provided for all employees.

Procedures

- Upon admission, the Direct Care Staff shall ensure that each resident receives a written copy of their rights within 24 hours. This shall be documented using the Resident Rights Acknowledgment Form.
- The Administrator shall post a visible copy of the Residents' Rights and complaint procedures in common areas immediately after the policy adoption. Compliance will be verified through a Compliance Check List.
- Training sessions on resident rights shall be conducted annually for all staff members, overseen by the Clinical Manager. Documentation of attendance shall be maintained through the Training Attendance Record.
- Supervisors shall facilitate regular resident meetings on a quarterly basis to discuss rights and gather feedback. Minutes of these meetings shall be meticulously recorded as Meeting Minutes.
- The Administrator shall review and update policies and training materials on an annual basis or as regulations change, with all updates documented in the Policy Review Document.
- The Administrator is responsible for ensuring compliance with state and federal regulations and must approve any updates to the policy and training materials. Additionally, they will monitor the implementation of resident rights.
- The Clinical Manager will oversee staff training and competency verification, ensuring quality of care relating to resident rights and responding to grievance issues as necessary.
- Supervisors are tasked to supervise daily operations to ensure respect for resident rights, provide ongoing support and training for direct care staff, and document resident feedback and concerns.
- Direct Care Staff must educate residents on their rights to support their autonomy and report any violations of resident rights promptly, maintaining respectful and professional communication throughout.
- To monitor compliance, the Quality Assurance Officer shall conduct regular audits and review resident feedback, with audits occurring quarterly. Results will be reported to the Administrator and discussed in team meetings.

- For any identified deficiencies, the Quality Assurance Officer will implement corrective action plans and monitor progress diligently.
- All staff must undergo comprehensive orientation training that includes resident rights training during initial onboarding, followed by annual refresher courses on resident rights and protections.
- Competency verification shall involve assessing staff through observational evaluations and collecting feedback from residents to ensure adherence to policy standards.
- Training records shall be maintained for audit purposes, ensuring accountability and compliance within the agency.

References

- **OAC 340:100-5-22.1**: Oklahoma Administrative Code on Resident Rights (link)
- **42 CFR 483.420**: CFR on Resident Rights for ICF/IID (link)

Therapy and Clinical Services Policy

State: `OAC 340:100-5-22, OAC 317:30-5-95` **Federal:** `42 CFR 483.440(b)-(d)`

Policy Statement

The Therapy and Clinical Services Policy at [AGENCY NAME] is designed to ensure that comprehensive therapy and clinical services are provided in strict compliance with all applicable state and federal regulations. The primary purpose of this policy is to safeguard the safety, rights, and individualized care of residents who require habilitative services. By establishing clear guidelines and procedures, this policy aims to fulfill our regulatory obligations while promoting best practices in the delivery and documentation of therapy services. [AGENCY NAME] shall implement a systematic approach to delivering therapy services that includes assessments, individualized planning, ongoing service provision, and thorough documentation. This policy reflects our commitment to quality care and the continuous improvement of our therapy services to meet the needs of each resident.

Procedures

- **Assessment:** Licensed Clinical Staff shall conduct an assessment of each resident to determine the need for therapy services. This assessment must occur within 3 days of admission. Documentation of the assessment will be recorded in the Assessment Report.
- **Individualized Program Plan (IPP):** The Clinical Manager is responsible for developing an Individualized Program Plan (IPP) that incorporates necessary therapy services as identified during the assessment. This plan shall be completed within 7 days of the assessment and documented accordingly.
- **Delivery of Therapy Services:** Therapists shall ensure that all therapy services are delivered in accordance with the established IPP and are documented within the resident's files. Proper documentation, including Session Notes, must be maintained during each therapy session.
- **Documentation Maintenance:** Records shall include Assessment Reports, Individualized Program Plans, and Therapy Session Notes. All documentation must be stored securely and confidentially for a minimum retention period of 5 years. The relevant forms, such as the IPP Form and Assessment Form, must be utilized for documentation purposes.
- **Quality Monitoring:** Regular audits of therapy documentation and service delivery will be conducted by the Quality Assurance Team on a quarterly basis. Findings from these audits are to be reported to the Administrator and Clinical Manager. Should deficiencies be identified, corrective action plans must be developed and implemented promptly to address these concerns.
- **Initial Training:** All clinical staff are required to complete orientation training on the Therapy and Clinical Services Policy to ensure understanding and compliance with established protocols.
- **Ongoing Education:** Staff must participate in annual training sessions focused on updates to best practices and compliance related to therapy and clinical services.
- **Competency Verification:** Competency assessments will be conducted following the initial training and annually thereafter to ensure staff proficiency in therapy service delivery and documentation practices. Training records for all staff must be maintained diligently.
- **Staff Responsibilities:** The Administrator shall oversee the implementation of this policy and ensure compliance with all regulatory requirements. The Clinical Manager will supervise therapy

staff and monitor the quality of therapy services, while supervisors are tasked with training staff on proper documentation procedures and regularly reviewing therapy documentation.

- **Collaboration of Direct Care Staff:** Direct care staff are responsible for reporting any issues related to therapy services and assisting in the documentation process to support comprehensive and accurate record-keeping.

References

- **OAC 340:100-5-22**: Oklahoma Administrative Code on Therapy Services
- **42 CFR 483.440**: Conditions of Participation for Intermediate Care Facilities

Transportation and Community Access Policy

State: `OAC 340:100-5-33.1, OAC 317:30-5-95.59` **Federal:** `N/A`

Policy Statement

The Transportation and Community Access Policy has been established to ensure compliance with state regulations while promoting safe, reliable transport for clients of Oklahoma Residential Habilitation Centers. The policy recognizes the growing need for accessible transportation in fostering community integration and enhancing the quality of life for residents with disabilities. By adhering to the requirements outlined in OAC 340:100-5-33.1 and OAC 317:30-5-95.59, [AGENCY NAME] is committed to protecting the rights of individuals. The agency shall facilitate transportation services that emphasize safety, dignity, and respect. [AGENCY NAME] shall provide safe, accessible transportation options to all clients while complying with state and federal laws. This policy is applicable to all personnel engaged in the transport of clients and mandates adherence to specific operational procedures. Ensuring effective transportation solutions will be reinforced through appropriate training for all relevant staff.

Procedures

- **Driver Verification:** The Transportation Coordinator shall verify that all drivers have completed the required background checks and training before assigning them to transport clients. Documentation of driver training records and background check results must be maintained.
- **Vehicle Maintenance:** The Facilities Manager shall conduct monthly vehicle maintenance inspections. Maintenance logs and inspection checklists must be duly completed and kept on file.
- **Transportation Documentation:** Direct Care Staff must ensure that all rides are documented immediately post-transport with client names, destinations, and times. This documentation will include mileage logs and transportation logs to track the services provided.
- **Feedback Review:** The Program Director shall review and analyze feedback from clients and families regarding transportation services on a quarterly basis. Summaries of feedback and action plans based on this feedback must be documented and addressed to improve service delivery.
- **Monitoring Compliance:** The Quality Improvement Officer shall conduct biannual audits to ensure compliance with this policy. Audit results will be reported to the agency leadership team, and immediate corrective actions will be implemented for any identified deficiencies.
- **Initial Training:** All staff must complete initial training on transportation policies and procedures within the first month of employment. Documentation of training must be maintained on file for review, ensuring all employees are adequately informed of their responsibilities in this area.
- **Ongoing Training:** Staff shall participate in annual refresher training on transportation safety and compliance. Competency verification must be conducted through a checklist to demonstrate proficiency in these areas, with training records documented for accountability.
- **Staff Responsibilities:**
    - Administrator: Approve policy and ensure compliance with state regulations; oversee training and implementation; address complaints regarding transportation services.
    - Clinical Manager: Ensure transportation services align with client care plans; evaluate client needs and individual transportation requirements.
    - Supervisors: Monitor the day-to-day operations of transportation staff; ensure proper documentation is maintained.

    - Direct Care Staff: Assist clients with transportation needs; document all transport services and incidents.

- **Record Keeping:** Records such as driver training records, client transportation logs, and vehicle maintenance records must be securely stored with access limited to authorized personnel. These records need to be retained for a period of 5 years to ensure compliance with regulations.

References

- **OAC 340:100-5-33.1**: Oklahoma Administrative Code on Transportation
- **OAC 317:30-5-95.59**: Oklahoma Residential Habilitation Centers Compliance Requirements

Documentation & Records Policies

Section Overview

The "Documentation & Records Policies" section outlines essential guidelines to ensure regulatory compliance and maintain the integrity of records within Oklahoma Residential Habilitation Centers. These policies focus on critical areas such as data retention and destruction, rights notification and documentation, and the management of clinical records. By adhering to these policies, agencies can effectively safeguard the rights of individuals in their care, ensure proper documentation practices, and facilitate transparent communication. Furthermore, robust documentation practices are vital for achieving and maintaining licensing and accreditation standards, which ultimately enhance service quality and accountability within the organization.

Data Retention and Destruction Policy

State: `OAC 340:100-3-39` **Federal:** `45 CFR 164.530`

Policy Statement

This Data Retention and Destruction Policy establishes guidelines to ensure that the records of residents and staff at residential habilitation centers in Oklahoma are managed in compliance with state and federal regulations. The policy aims to protect the confidentiality of sensitive information while also ensuring that [AGENCY NAME] maintains accurate records for accountability and regulatory compliance. By safeguarding the rights of residents, this policy promotes trust in the services provided by [AGENCY NAME]. [AGENCY NAME] shall ensure that all data is retained as required by law, securely stored, and properly destroyed once the retention period has expired. This policy applies to all employees, contractors, and third-party vendors involved in the handling of records at [AGENCY NAME]. Compliance with OAC 340:100-3-39 and HIPAA regulations is compulsory; failure to adhere to these requirements may result in corrective actions.

Procedures

- **Identify and Categorize Data**: The Data Management Officer shall identify and categorize all types of data collected, including personal information, medical records, and incident reports, within 30 days of policy implementation. An inventory report documenting these data categories must be maintained. - **Establish Retention Timeframes**: Within 60 days of policy implementation, the Data Management Officer shall establish retention timeframes for each data category according to OAC and HIPAA guidelines. This will be documented in a retention schedule document. - **Implement Secure Storage Procedures**: The IT Manager shall implement secure storage procedures for all retained data, ensuring confidentiality and access control. Proper storage solutions, both electronic and physical, must be employed, with physical records locked in secure cabinets. Ongoing documentation must be maintained in a storage security protocol log. - **Regular Data Reviews**: The Compliance Officer shall schedule annual reviews of retained data to determine necessity and relevance. These reviews will result in a data review report that confirms continued compliance with retention requirements. - **Document Destruction of Records**: The Data Management Officer shall document the secure destruction of records that are no longer needed. Each instance of destruction must be logged, acknowledging methods and dates, ensuring accountability throughout the process. - **Monitoring Compliance**: The Compliance Officer will conduct regular audits and reviews of the records management system for compliance, with a frequency of quarterly evaluations. Results of these monitoring activities shall be documented and reported to the Administrator. Immediate corrective measures must be taken for any non-compliance discovered during audits. - **Training Requirements**: All staff must undergo initial training on the Data Retention and Destruction Policy during their orientation. Annual refresher training will cover updates to relevant legislation and internal policies. Competency verification will occur through post-training assessments to ensure understanding of data handling procedures. Attendance records for all training sessions must be maintained for auditing purposes. - **Staff Responsibilities**: - The Administrator shall approve and oversee the implementation of this policy, ensuring adequate funding and resources for compliance. - The Clinical Manager shall perform regular audits to verify staff compliance with data handling procedures and provide training on policy updates. - Supervisors shall monitor daily compliance with retention and destruction procedures, reporting any discrepancies or issues with data management. - Direct Care Staff shall follow established procedures for data handling and immediately report any breaches while participating in required training sessions.

References

- **OAC 340:100-3-39**: Oklahoma Administrative Code - Data Retention and Destruction Requirements (link)
- **45 CFR 164.530**: HIPAA Regulations - Administrative Requirements (link)

Rights Notification and Documentation Policy

State: `OAC 340:100-5-22.1, OAC Chapter 680` **Federal:** `N/A`

Policy Statement

The Rights Notification and Documentation Policy exists to ensure compliance with state and federal regulations regarding the rights of residents in Oklahoma Residential Habilitation Centers. This policy provides a structured approach to informing residents of their rights, obtaining consent, and documenting their understanding of these rights. It aims to enhance safety, quality of care, and the empowerment of residents by making them aware of their rights. The policy applies to all staff members and residents within the facilities governed by this regulation. [AGENCY NAME] shall uphold the rights of residents as ensured by OAC 340:100-5-22.1 and align with federal guidelines, including the CMS standards. This includes a commitment to provide necessary training for staff regarding these rights, ensuring clear communication, and creating an abuse-free environment. In order to ensure that all residents are informed of their rights and that staff are equipped to uphold and respect these rights, [AGENCY NAME] will implement a series of procedures aimed at fostering a respectful and compliant environment. This policy mandates a systematic approach beginning at the time of admission and continuing through annual reviews, all firmly grounded in robust documentation and training efforts.

Procedures

- **Initial Rights Notification:**
  - The Admissions Coordinator will provide each resident with a written statement of their rights within 24 hours of admission. This documentation shall be recorded on the Resident Rights Acknowledgment Form.
  - Within 48 hours of admission, Direct Care Staff will meet with each resident to review and explain their rights in a clear and understandable manner. The discussion shall be documented as a Documentation of Rights Discussion.
  - During this rights discussion, Direct Care Staff will collect signed consent forms from residents, acknowledging their understanding of their rights. This shall be recorded on a Signed Consent Form.
- **Annual Review of Rights:**
  - The Administrator shall ensure that a written statement of rights is re-issued to all residents annually. Furthermore, a review of these rights shall be conducted, with documentation maintained via the Annual Rights Mailer.
- **Ongoing Compliance Monitoring:**
  - The Quality Assurance Coordinator will conduct regular audits and resident feedback sessions to monitor adherence to the rights notification procedures. This shall occur quarterly, with findings reported to the Administrator and staff.
  - Immediate corrective action is required for compliance issues identified during audits to ensure residents' rights are consistently upheld.

- **Staff Training Requirements:**
  - All staff members must complete training on resident rights and documentation standards within 30 days of employment. This initial training must cover both the content of the rights and documentation requirements.
  - Annual training refreshers are required to reinforce knowledge and ensure compliance with updated policies and regulations.
  - Staff competency shall be verified through training quizzes and practical evaluations, with records of training completion maintained in personnel files.
- **Documentation Retention:**
  - All documentation related to residents' rights notification, including the Resident Rights Acknowledgment Form, Documentation of Rights Discussion, Signed Consent Form, and Annual Rights Mailer, shall be retained for a period of five years.
  - All documentation must be stored securely and accessible only to authorized personnel to maintain confidentiality and compliance.

References

- **OAC 340:100-5-22.1**: Oklahoma Administrative Code on Rights Notification
- **42 CFR 483.400-483.480**: Conditions of Participation for ICF/IID
- **ADA Title III**: Americans with Disabilities Act Standards (link)

Clinical Records and Documentation Policy

State: `OAC 340:100-3-39` **Federal:** `42 CFR 483.420(c)`

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Policy Statement

This Clinical Records and Documentation Policy exists to ensure compliance with both state and federal regulations concerning the documentation of clinical records in residential habilitation centers. The goal is to promote the safety and welfare of clients by maintaining thorough, accurate, and accessible documentation regarding each client's medical and service records. By establishing a standardized approach to documentation, this policy protects the rights of clients, supports continuity of care, and enhances the quality of services provided. [AGENCY NAME] shall ensure that all clinical records are maintained accurately and securely, accessible to authorized personnel, and completed in a timely manner following federal and state guidelines. All staff members at [AGENCY NAME] must adhere strictly to the outlined procedures to uphold compliance and protect the rights of clients. This policy covers all clinical records and documentation practices for clients served in residential habilitation settings, emphasizing the importance of accurate and timely documentation in the delivery of quality services.

Procedures

- **Initial Assessment:** Direct Care Staff shall conduct an initial assessment of the client's medical history and current needs within 24 hours of admission. This assessment will be documented using the Client Assessment Document.
- **Individual Program Plan (IPP) Development:** The Clinical Manager shall be responsible for developing and implementing an Individual Program Plan (IPP) for each client within 5 days of admission. Documentation shall be maintained in the Client IPP Document.
- **Updating Clinical Records:** Immediately following each service delivery, Direct Care Staff shall review and update the client's clinical records using the Service Delivery Documentation Form. All entries must be signed and dated by the responsible staff member at the time of entry.
- **Regular Audits:** The Administrator shall conduct quarterly audits of clinical records for compliance, with findings documented in an Audit Report. These audits aim to ensure ongoing adherence to documentation standards and identify any necessary corrective actions.
- **Quality Assurance Practices:** Supervisors will facilitate daily implementation of documentation practices and review staff documentation for quality assurance, ensuring accuracy and compliance with established standards.
- **Training Requirements:** All staff must complete initial training on clinical documentation practices during orientation. Ongoing education will be provided through annual training refreshers on documentation standards and compliance. Documentation of all training records must be maintained and updated annually.
- **Competency Verification:** Competency shall be assessed through annual reviews and documentation audits, ensuring that staff are properly trained and compliant with the Clinical Records and Documentation Policy.
- **Record Retention and Storage:** Records must include client assessments, Individual Program Plans (IPPs), progress notes for each service provided, and incident reports when necessary. All

records shall be securely stored to ensure confidentiality and compliance with HIPAA requirements, with a retention period of 5 years.

- **Addressing Discrepancies:** Direct Care Staff are responsible for reporting any discrepancies or compliance issues to a supervisor to facilitate immediate resolution and uphold the integrity of documentation processes.

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### References

- **OAC 340:100-3-39**: Oklahoma Administrative Code for Clinical Records
- **42 CFR 483.420(c)**: Federal Regulation on Clinical Records
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Financial & Billing Policies

Section Overview

The "Financial & Billing Policies" section is designed to ensure compliance with regulatory requirements while safeguarding the financial integrity of Oklahoma Residential Habilitation Centers. This section encompasses two primary policies: the Billing, Claims, and Cost Reporting Policy, which outlines procedures for accurate billing and reimbursement, and the Resident Funds Protection and Management Policy, which focuses on the safeguarding and responsible management of residents' funds. Key focus areas for Residential Habilitation Centers agencies include transparency, accountability, and fiscal responsibility in financial practices. Adhering to these policies is crucial for maintaining licensing and accreditation, as they demonstrate a commitment to ethical financial management and the protection of residents' assets, thereby fostering trust and reliability within the community.

Billing, Claims, and Cost Reporting Policy

State: `OAC 340:100-3-60, OHCA Cost Report Guidelines` **Federal:** `N/A`

Policy Statement

The purpose of this policy is to establish a comprehensive framework for billing, claims processing, and cost reporting for Residential Habilitation Centers in Oklahoma as mandated by state regulations OAC 340:100-3-60. This policy aims to ensure compliance with state and federal guidelines, safeguard financial integrity, and enhance the quality of care for clients. [AGENCY NAME] is committed to protecting both its financial resources and the rights of its residents through clear guidelines and adherence to regulatory standards. In alignment with regulatory requirements, [AGENCY NAME] shall implement efficient billing practices that comply with all state and federal requirements, ensuring timely and accurate claims submissions. This policy applies to all staff involved in the billing and claims process and emphasizes the importance of ongoing compliance with applicable regulations.

Procedures

- **Prepare Initial Claims:** The Billing Specialist shall prepare initial claims for Medicaid reimbursement within 5 business days after service delivery. Required documentation includes the claim submission form and service logs.
- **Monthly Review of Denied Claims:** The Billing Manager must conduct a monthly review of claims denied by payers to identify trends and facilitate corrective actions. This process will utilize denial reports and an analysis summary for documentation.
- **Quarterly Cost Reporting:** The Administrator shall submit quarterly cost reports to the Oklahoma Health Care Authority as per OHCA guidelines, with reports due by the 15th of the month following the end of the quarter. Documentation required includes cost report forms.
- **Annual Review of Billing Procedures:** The Policy Coordinator is responsible for reviewing and updating billing procedures and protocols annually or as necessary to reflect changes in regulations. The revised policy document will serve as the official record of updates.
- **Administrator Responsibilities:** The Administrator must ensure overall compliance with billing and claims regulations and approve cost reports prior to submission to regulatory agencies.
- **Compliance Verification by Clinical Manager:** The Clinical Manager shall verify service delivery records for compliance with Medicaid regulations and train direct care staff on accurate documentation practices.
- **Billing Operations Monitoring:** Supervisors will monitor daily billing operations and staff adherence to protocols, providing assistance in resolving any billing discrepancies that arise.
- **Documentation by Direct Care Staff:** Direct Care Staff must document services provided to residents accurately and timely, reporting any issues related to service documentation immediately to their supervisor.
- **Record Maintenance:** Records such as service delivery logs, billing transaction records, and audit trails of claims submissions must be maintained for a retention period of 5 years. Secure electronic storage with restricted access will be utilized for all billing information.
- **Periodic Audits:** The Compliance Officer shall conduct quarterly audits of billing submissions and financial records, reporting findings to the Administrator and Board of Directors. Immediate

rectification of identified deficiencies will be necessary, along with subsequent follow-up audits to ensure compliance.

- **Initial Training for Billing Staff:** All billing staff must complete training on Medicaid billing procedures prior to assuming their duties to ensure competency in the required practices.
- **Ongoing Education:** Annual refresher training will be provided to all staff focusing on updated regulations and billing practices. Documentation of training attendance records and competency assessment results will be maintained.
- **Competency Verification:** Staff competencies will be evaluated through practical assessments and audits to ensure ongoing compliance and proficiency in billing procedures.

References

- **OAC 340:100-3-60**: Oklahoma Administrative Code on Billing and Cost Reporting (link)
- **CMS State Operations Manual**: Conditions of Participation for ICFs/IID (link)

Resident Funds Protection and Management Policy

State: `OAC 340:100-3-8.1` **Federal:** `42 CFR 483.420(a)(5)`

Policy Statement

The Resident Funds Protection and Management Policy is established by [AGENCY NAME] to ensure the safeguarding and responsible management of resident funds as mandated by OAC 340:100-3-8.1 and 42 CFR 483.420(a)(5). This policy is designed to protect the financial interests of residents within residential habilitation centers, thereby supporting their dignity and independence. [AGENCY NAME] is committed to compliance with all regulatory standards, promoting the delivery of high-quality services, and preventing financial exploitation of residents. This policy applies to all employees, agents, and representatives of [AGENCY NAME]. The key obligations set forth in this policy include protecting residents' funds from mismanagement, ensuring a clear and transparent process for fund management and expenditures, maintaining accurate financial records, and providing residents with access to their financial information.

Procedures

- **Verify resident identity**
  - Action: Verify resident identity before accepting any funds.
  - Responsible: Intake Coordinator
  - Timing: At admission
  - Documentation: Maintain identity verification records to confirm the authenticity of each resident enrolled.
- **Document transactions**
  - Action: Document all transactions concerning resident funds in the financial management system.
  - Responsible: Financial Officer
  - Timing: Immediately after each transaction
  - Documentation: Maintain an up-to-date transaction log that captures all financial activities related to resident funds.
- **Conduct internal audits**
  - Action: Conduct internal audits of resident accounts annually.
  - Responsible: Compliance Officer
  - Timing: Annually
  - Documentation: Compile an audit report that outlines the findings and compliance with resident funds management policies.
  - Quality Monitoring: Audit results will be reported to the agency board, and corrective measures shall be implemented immediately for any discrepancies identified.
- **Provide account statements**
  - Action: Provide residents with a quarterly statement of their account.
  - Responsible: Financial Officer

- Timing: Quarterly
- Documentation: Maintain quarterly financial statements accessible to residents to promote transparency and trust.

- **Staff training**
  - Action: Ensure all staff complete training on resident funds management during orientation.
  - Timing: Initial training upon hiring and annual refresher training thereafter.
  - Competency Verification: Assess competency through quarterly scenarios and tests to ensure staff understand financial policies.
  - Documentation: Maintain training records for all staff to document compliance with training requirements.

- **Monitoring Procedures**
  - Action: Implement ongoing monitoring of financial transactions to ensure compliance with policies.
  - Responsible: Compliance Officer
  - Frequency: Annual audits and continuous transaction monitoring as part of daily operations.

- **Staff Responsibilities**
  - Administrator: Ensure overall compliance with state and federal regulations and approve the resident funds management policy.
  - Clinical Manager: Oversee staff training regarding funds management and monitor adherence to financial policies.
  - Supervisors: Ensure staff follow procedures for handling resident funds and review transaction logs weekly for discrepancies.
  - Direct Care Staff: Assist residents with accessing their funds per policy and report any suspicious financial activities immediately.

References

- **OAC 340:100-3-8.1**: Oklahoma Administrative Code - Resident Funds Management
- **42 CFR 483.420(a)(5)**: Code of Federal Regulations - Conditions of Participation

Infection Control Policies

Section Overview

The Infection Control Policies section outlines essential guidelines designed to ensure the health and safety of residents within Oklahoma Residential Habilitation Centers. These policies, including the Immunization and Infectious Disease Policy, Infection Prevention and Control Policy, Exposure Control and Reporting Policy, and Standard and Transmission-Based Precautions Policy, serve to comply with regulatory standards and to mitigate the risk of infectious diseases. Key focus areas include the implementation of immunization protocols, effective infection surveillance, systematic reporting of exposure incidents, and adherence to precautionary measures. By rigorously following these policies, Residential Habilitation Centers not only protect the well-being of residents and staff but also fulfill licensing and accreditation requirements, ultimately enhancing the quality of care provided within these facilities.

Immunization and Infectious Disease Policy

State: `OAC 340:100-3-22.1` **Federal:** `CMS Infection Control`

Policy Statement

The Immunization and Infectious Disease Policy exists to comply with Oklahoma state regulations and federal CMS Infection Control guidelines, ensuring the health and safety of both residents and staff within residential habilitation centers. This policy aims to facilitate timely immunizations for staff and residents alike, thereby minimizing the risk of infectious disease transmission. By implementing robust compliance mechanisms, the agency emphasizes proactive health measures that safeguard vulnerable populations while adhering to all necessary legal requirements. [AGENCY NAME] commits to ensuring a systematic approach that promotes immunization awareness, education, and compliance. The agency shall uphold the highest standards of care in immunization practices, thereby protecting the health of its residents and workforce.

Procedures

- Upon admission of each resident, the nursing staff shall assess their immunization needs within 48 hours. This assessment will be documented using the immunization assessment form, which must be completed and filed appropriately. This process will be repeated annually thereafter.
- Prior to administering any vaccines, nursing staff must document parental or guardian consent for immunizations. A signed and dated consent form must be obtained and maintained in the resident's medical record.
- Routine immunizations will be administered by a qualified healthcare provider according to established state and federal guidelines. The vaccination record shall be updated immediately following the administration of any vaccine.
- Nursing staff shall monitor and document any side effects or adverse reactions to vaccines immediately after vaccination and for a period of 24 hours post-administration. This information must be recorded in the adverse reaction log to ensure comprehensive tracking and reporting.
- Nursing staff or designated staff trainers will regularly educate residents and families about the benefits and potential risks associated with vaccines. This education must take place at least once per year, with documentation of materials distributed and sessions conducted maintained in the training records.
- Human Resources or the Nursing Supervisor will conduct an annual review and verification of staff immunization status, completing the staff immunization tracking log to ensure compliance. This process will occur upon hiring and annually thereafter.
- The Quality Assurance Officer shall conduct quarterly reviews of immunization records during compliance checks. Any deficiencies identified will require immediate corrective action, with findings reported to the Administrator during staff meetings.
- All staff shall complete initial training on immunization policies during orientation. Subsequent ongoing education, including annual refresher training on immunization practices and consent protocols, will be mandatory. Competency assessments shall be documented following the completion of this training, and training attendance records will be maintained in personnel files to ensure accountability.

References

- **OAC 340:100-3-22.1**: Oklahoma Administrative Code on Immunization Requirements (link)
- **CMS Infection Control**: Centers for Medicare & Medicaid Services Infection Control Guidelines (link)

Infection Prevention and Control Policy

State: `OAC 340:100-3-22.1` **Federal:** `42 CFR 483.470(i)`

Policy Statement

The Infection Prevention and Control Policy of [AGENCY NAME] is essential to safeguarding the health and safety of all residents. This policy ensures compliance with applicable state and federal regulations, including OAC 340:100-3-22.1 and 42 CFR 483.470(i). Recognizing the potential risks associated with infections, this policy actively promotes a healthy and safe environment for both residents and staff through established procedures and guidelines aimed at preventing and controlling infections. [AGENCY NAME] is committed to implementing rigorous infection prevention practices that incorporate thorough assessments, continuous monitoring, and staff training. This commitment is fundamental to mitigating infection risks and fostering a culture of safety throughout our facilities.

Procedures

- **Conduct a thorough infection risk assessment for each resident.**
  - **Responsible:** Clinical Manager
  - **Timing:** Upon admission and annually thereafter
  - **Documentation:** Infection Risk Assessment Form
- **Implement infection control practices based on assessment findings.**
  - **Responsible:** Direct Care Staff
  - **Timing:** Ongoing as per risk assessment
  - **Documentation:** Infection Control Implementation Log
- **Provide training on infection prevention to all staff.**
  - **Responsible:** Training Coordinator
  - **Timing:** At orientation and annually
  - **Documentation:** Training Attendance Record
- **Monitor and record infection rates among residents weekly.**
  - **Responsible:** Quality Assurance Coordinator
  - **Timing:** Weekly
  - **Documentation:** Infection Rate Monitoring Report
  - **Method:** Weekly audits of infection control practices
  - **Reporting:** Results to be reported to the Administrator and staff
  - **Corrective Action:** Non-compliance issues to be addressed within 48 hours
- **Review and update the infection prevention policy annually.**
  - **Responsible:** Administrator
  - **Timing:** Annually
  - **Documentation:** Policy Review Document

- **Assess and maintain staff competency in infection control protocols.**
  - **Training:** Initial training during orientation and annual refresher training
  - **Competency Verification:** Assessment through knowledge quizzes and practical demonstrations
  - **Documentation:** All training must be maintained in employee files

This comprehensive set of procedures ensures that [AGENCY NAME] not only meets regulatory requirements but also fosters a safe and healthy environment for both residents and staff. Each team member plays a crucial role in adhering to these protocols, reinforcing our commitment to infection prevention and control in all our operations.

References

- **OAC 340:100-3-22.1**: Oklahoma Administrative Code - Infection Control (link)
- **42 CFR 483.470(i)**: Federal Regulation - Infection Control (link)
- **CMS Infection Control**: Centers for Medicare & Medicaid Services - Infection Control Guidelines (link)

Exposure Control and Reporting Policy

State: `OAC 340:100-3-22.1` **Federal:** `N/A`

Policy Statement

This Exposure Control and Reporting Policy is established to ensure compliance with state and federal regulations regarding exposure control, protecting the health and safety of both residents and staff from bloodborne pathogens. [AGENCY NAME] is committed to actively managing exposure risks, implementing robust reporting protocols, and promoting a safe environment within all Residential Habilitation Centers. This policy explicitly applies to all staff and residents involved in center operations and adheres to the standards set forth in OAC 340:100-3-22.1. To achieve these objectives, [AGENCY NAME] shall implement thorough procedures for identifying potential exposure risks and conducting regular training for all staff members. Continuous improvement and compliance with relevant regulations are paramount to our mission of ensuring safety and well-being for everyone within our care.

Procedures

- **Annual Risk Assessment:** Conduct comprehensive risk assessments to identify possible sources of exposure to bloodborne pathogens. This task is the responsibility of the Safety Officer and should be executed annually. The findings must be documented in a risk assessment report for review.
- **Implementation of Precautions:** Based on the identified risks, the Clinical Manager shall implement necessary precautions, including the proper use of personal protective equipment (PPE), before the assignment of any staff. Documentation of training completion for PPE usage must be maintained.
- **Training for Staff:** All staff members must receive training on exposure risks and reporting procedures at orientation and at least annually thereafter. The Training Coordinator will oversee this training, and training attendance records must be documented for compliance purposes.
- **Reporting Exposure Incidents:** Direct Care Staff are required to report any exposure incidents immediately, following the established reporting processes. Each incident must be documented using the designated incident report form.
- **Compliance Oversight:** The Administrator shall ensure policy compliance and make necessary updates as required, while also overseeing the implementation of exposure control measures. Regular audits will be conducted by the Clinical Manager to ensure that clinical staff adheres to exposure protocols.
- **Staff Monitoring:** Supervisors must train staff on the policy and reporting procedures and continuously monitor compliance, addressing any issues that arise in a timely manner.
- **Retention of Documentation:** All documentation, including risk assessments, incident reports, and training attendance records, must be securely stored and accessible only to authorized personnel. These records shall be retained for a minimum of five years.
- **Quarterly Audits:** The Quality Assurance Manager shall conduct regular audits and reviews of training records and incident reports on a quarterly basis. Findings from these audits must be reported to the Administrator and utilized to inform ongoing training updates.
- **Corrective Actions:** Should any deficiencies be identified during audits, corrective actions must be implemented, including retraining staff and making necessary modifications to the policy.

- **Ongoing Training:** All new staff shall receive initial training focused on exposure risks and reporting procedures during orientation. Furthermore, annual training sessions covering updates to this Exposure Control Policy are mandatory. Staff must demonstrate their understanding through practical assessments during the annual training, with all attendance and competency assessment records documented.

References

- **OAC 340:100-3-22.1**: Oklahoma Administrative Code for Exposure Control
- **OSHA Bloodborne Pathogens Standard**: OSHA Regulation (link)

Standard and Transmission-Based Precautions Policy

State: `CMS Infection Control` **Federal:** `N/A`

Policy Statement

This Standard and Transmission-Based Precautions Policy is established to ensure compliance with state and federal regulations pertaining to infection control, thereby enhancing the safety and quality of care provided to residents in Oklahoma Residential Habilitation Centers. The overarching aim is to mitigate the risk of infection transmission, ensuring the health and well-being of both residents and staff. [AGENCY NAME] is dedicated to fostering a safe healthcare environment by implementing and maintaining effective infection control practices to prevent the spread of infectious diseases. To this end, this policy applies to all staff, residents, and visitors within the facility. [AGENCY NAME] shall adhere to all relevant state and federal regulations, reinforcing its commitment to provide a safe environment free from the risk of infection. Furthermore, compliance with the outlined practices is the collective responsibility of all staff members.

Procedures

- **Conduct a risk assessment** to identify areas at risk of infection within the facility.
  - **Responsible:** Infection Control Coordinator
  - **Timing:** Annually or as needed when significant changes occur in the facility
  - **Documentation:** Risk assessment report
- **Train staff** on Standard and Transmission-Based Precautions during their initial orientation.
  - **Responsible:** Training Coordinator
  - **Timing:** Before direct assignment to patient care responsibilities
  - **Documentation:** Training attendance records
- **Implement and monitor hand hygiene protocols**, ensuring hand sanitizers are readily accessible in all units.
  - **Responsible:** Direct Care Supervisors
  - **Timing:** Ongoing, with monthly audits
  - **Documentation:** Audit records of hand hygiene compliance
- **Assess and provide appropriate personal protective equipment (PPE)** for staff based on the risk assessment.
  - **Responsible:** Infection Control Coordinator
  - **Timing:** At the time of risk assessment and reassessed quarterly
  - **Documentation:** PPE inventory records
- **Conduct regular infection control audits** to ensure compliance with this policy.
  - **Responsible:** Quality Assurance Team
  - **Timing:** Quarterly
  - **Documentation:** Infection control audit reports
  - **Monitoring Method:** Results reported to the administration and shared with relevant staff

    - **Corrective Action:** Develop and implement corrective action plans based on audit findings

- **Establish isolation protocols** for residents with known or suspected infections.
    - **Responsible:** Clinical Manager
    - **Timing:** Ongoing, reviewed during resident changes in health status
    - **Documentation:** Isolation protocol logs

- **Provide ongoing education** through annual refresher training on infection control practices and updates.
    - **Competency Verification:** Competency assessed through direct observation and audits
    - **Documentation:** Training logs maintained for all staff

Responsibilities

- **Administrator:**
    - Oversee compliance with infection control policies
    - Ensure resources are available for implementing infection control measures

- **Clinical Manager:**
    - Supervise clinical staff to ensure adherence to infection control procedures
    - Provide clinical guidance on infection control

- **Supervisors:**
    - Train staff on procedures related to infection control
    - Monitor compliance and document training

- **Direct Care Staff:**
    - Implement Standard and Transmission-Based Precautions
    - Document adherence to practices and report any breaches

References

- **OAC 340:100**: Oklahoma Administrative Code for Residential Habilitation Centers
- **42 CFR 483.400-483.480**: Conditions of Participation for ICF/IID (link)

Personnel & Staffing Policies

Section Overview

The "Personnel & Staffing Policies" section establishes essential guidelines to ensure regulatory compliance and promote a safe, effective environment in Oklahoma Residential Habilitation Centers. These policies focus on critical areas such as comprehensive staff training, ongoing supervision, performance evaluations, adequate staffing coverage, and thorough background checks, all of which serve to protect the well-being of residents and enhance care quality. By adhering to these policies, agencies demonstrate their commitment to maintaining high professional standards, which is crucial for securing and retaining necessary licensing and accreditation. Ultimately, these policies aim to foster a skilled and responsive workforce dedicated to delivering the highest level of support to individuals in residential habilitation settings.

Staff Training and Orientation Policy

State: `OAC 340:100-3-34, OAC 340:100-5-35` **Federal:** `N/A`

Policy Statement

The purpose of the Staff Training and Orientation Policy is to ensure that all personnel at residential habilitation centers in Oklahoma receive comprehensive training and orientation. This initiative is vital for adhering to regulatory requirements as set forth by the Oklahoma Administrative Code, and to promote the safety and welfare of the clients we serve. This policy underscores the importance of enhancing staff competencies, which are critical for providing quality care and support to individuals with developmental disabilities. [AGENCY NAME] is dedicated to implementing effective training protocols to align with OAC 340:100-3-34 and OAC 340:100-5-35. This policy applies to all personnel involved in the direct care and support of residents. Our commitment is to ensure that all staff fulfill their training obligations to safeguard client welfare and meet the demands of our regulatory framework.

Procedures

- **Initial Training:**
  - All new staff shall complete the Oklahoma DDS core training curriculum prior to their direct assignment to residents. This curriculum includes foundational training, positive behavior support, and incident reporting. The Training Coordinator will oversee this process, and training completion records must be maintained.
  - Upon hire, each staff member shall participate in emergency procedures training, covering fire and disaster response protocols, within the first month of employment and annually thereafter. The Safety Officer is responsible for conducting this training, with attendance records documented for compliance.

- **Ongoing Training and Competency Verification:**
  - All direct care staff shall engage in ongoing training annually, focusing on new regulations, policies, and refresher courses pertinent to their roles. Records of all training sessions must be maintained.
  - Competency will be assessed through periodic evaluations, practical exams, and performance reviews administered annually by Supervisors, who are responsible for implementing daily training activities and ensuring staff participation.
  - Performance review documents, detailing evaluations of competencies, shall be completed and retained for all direct care staff each year.

- **Documentation Requirements:**
  - Training attendance logs, competency assessment results, and performance review documents must be securely stored for a retention period of five years, ensuring restricted access for confidentiality.
  - Documentation layouts must conform to Oklahoma's specific requirements for training records, including the use of Training Completion Forms and Incident Report Forms to track training efficacy and incident management.

- **Monitoring Compliance:**
  - The Quality Assurance Officer shall conduct regular audits of training records and compliance checks quarterly. Findings from these audits will be reported to the Administrator, along with any needed corrective actions based on identified deficiencies.
  - If deficiencies are identified, a corrective action plan must be implemented promptly to ensure compliance and maintain high-quality standards in staff training and resident care.

References

- **OAC 340:100-3-34**: Oklahoma Administrative Code for Personnel Training Requirements (link)
- **OAC 340:100-5-35**: Oklahoma Administrative Code for Specialized Training Standards (link)

Staff Supervision and Performance Evaluation Policy

State: `OAC 340:100-3-34` **Federal:** `42 CFR 483.430(e)(1)-(2)`

Policy Statement

The purpose of this policy is to establish a framework for staff supervision and performance evaluation within [AGENCY NAME]. This policy exists to ensure compliance with applicable state and federal regulations, specifically OAC 340:100-3-34 and 42 CFR 483.430. By providing structured oversight of staff performance, this policy aims to protect clients, staff, and the organization while promoting continuous improvement in care delivery. [AGENCY NAME] is committed to maintaining the highest standards of care through a systematic approach to performance evaluation. This policy mandates a comprehensive process that includes clear criteria for performance assessments, regular evaluations, and actionable feedback. It applies to all employees involved in client care, ensuring that evaluations are conducted fairly and consistently while fostering a culture of professional development and accountability.

Procedures

- The **Administrator** shall oversee the implementation of the Staff Supervision and Performance Evaluation Policy, ensuring compliance with OAC 340:100-3-34 and 42 CFR 483.430. This includes reviewing evaluation results for all staff members annually, prior to the end of the fiscal year. Documentation will comprise a completed and signed performance evaluation form for each staff member.
- Prior to the performance evaluation period each year, the **Administrator** will provide training on evaluation criteria and procedures to all supervisory staff. An attendance sheet for the training must be documented to verify participation.
- **Supervisors** shall conduct annual performance evaluations for all staff based on established criteria. They are responsible for providing necessary training and support to direct care staff, as well as maintaining thorough documentation of evaluations. Completed evaluation forms must be signed by both the evaluator and the staff member, ensuring accountability.
- The **Clinical Manager** shall monitor clinical quality during evaluations, ensuring that training for evaluators remains current. Support will be provided to supervisors to facilitate accurate and effective evaluations.
- All **Direct Care Staff** are required to participate in performance evaluations and must engage in professional development opportunities to enhance their skills and competencies.
- The **Quality Assurance Coordinator** shall conduct internal audits of performance evaluation documentation and compliance checks on a quarterly basis. Findings from these audits will be reported to the Administrator and presented at the quarterly leadership meeting. In the event deficiencies are identified, corrective actions shall be developed, including an action plan and retraining for involved staff.
- Initial training on performance evaluation procedures must be completed by all supervisory staff during their introductory training phase. Ongoing education will involve annual refresher training on performance evaluation criteria and methods. Competency verification will be conducted through assessments following training, ensuring staff are equipped to implement evaluation standards effectively.

- Records must be maintained for all training, encompassing dates, attendees, and content covered. This documentation will serve as a reference to ensure ongoing compliance and quality in the performance evaluation process. Records, including performance evaluations, training records, and incident reports related to staff conduct, must be securely stored for a retention period of five years, accessible only to authorized personnel.

References

- **OAC 340:100-3-34**: Oklahoma Administrative Code on Staff Supervision (link)
- **42 CFR 483.430**: Federal Regulation on Staff Supervision and Performance Evaluation (link)

Staffing Plan and Sufficient Coverage Policy

State: `OAC 340:100-3-34, OAC 340:100-5-35` **Federal:** `42 CFR 483.430(d)`

Policy Statement

[AGENCY NAME] shall establish a Staffing Plan and Sufficient Coverage Policy to ensure compliance with state and federal regulations governing residential habilitation centers. This policy aims to maintain a qualified workforce capable of meeting the needs of all residents, thereby promoting their safety, health, and overall quality of care. The policy exists to protect the rights of residents while ensuring that staffing levels are sufficient to deliver individualized services effectively. The agency commits to adhering to all relevant regulatory requirements while supporting the well-being of both staff and residents. The Staffing Plan shall be applicable to all employees and operations within [AGENCY NAME], covering aspects such as minimum staffing ratios, training protocols, documentation of staffing presence on shifts, and compliance monitoring procedures. Key obligations include the provision of sufficient staff per resident needs and regulatory standards, compliance with ongoing training and credential requirements, and assurance of regular assessment and documentation to track staffing effectiveness.

Procedures

- **Step 1:** Conduct a staffing needs assessment to determine required staffing ratios based on resident acuity.
  - **Responsible:** Clinical Manager
  - **Timing:** Annually or as changes in occupancy occur
  - **Documentation:** Staffing needs assessment report shall be maintained for compliance review.
- **Step 2:** Develop and maintain job descriptions for all staff positions, outlining necessary qualifications and responsibilities to ensure clarity in duties.
  - **Responsible:** Human Resources Manager
  - **Timing:** Within 30 days of position opening or changes
  - **Documentation:** Updated job descriptions shall be secured for reference.
- **Step 3:** Schedule and conduct the required training for all staff, ensuring training covers all mandated areas including regulations and care practices.
  - **Responsible:** Training Coordinator
  - **Timing:** Initial training upon hire and annually thereafter
  - **Documentation:** Training attendance records must be maintained and readily available for inspection.
- **Step 4:** Submit schedules of staff-to-resident ratios to the state as required for compliance.
  - **Responsible:** Administrator
  - **Timing:** Quarterly
  - **Documentation:** Submission confirmation documents must be retained in compliance records.

- **Step 5:** Audit staffing records and training compliance regularly to ensure ongoing adherence to regulations.
  - **Responsible:** Quality Assurance Coordinator
  - **Timing:** Bi-annually
  - **Documentation:** Audit reports shall be created and stored for both internal and external reviews.
- **Step 6:** Monitor compliance with staffing and training protocols through regular audits and compliance checks of staffing records and training.
  - **Frequency:** Quarterly and during annual inspections
  - **Responsible:** Quality Assurance Coordinator
  - **Reporting:** Findings must be reported to the Administrator and state authorities as necessary.
  - **Corrective Action:** Develop and implement a plan of correction for identified deficiencies to ensure continued compliance.
- **Step 7:** Ensure initial and ongoing training completion for all staff, including annual refresher training sessions on applicable regulations and care practices.
  - **Competency Verification:** Staff competencies will be assessed through regular performance evaluations and documentation of training attendance.
  - **Documentation:** Maintain up-to-date training logs for all staff to ensure accountability and compliance.

References

- **OAC 340:100-3-34**: Oklahoma Administrative Code on Personnel (link)
- **42 CFR 483.430**: Federal Regulation on Staffing Requirements (link)

Employee Criminal Background Check and Exclusion Screening Policy

State: `OAC 340:100-3-34, OAC Chapter 680:3-10` **Federal:** `N/A`

Policy Statement

The [AGENCY NAME] is committed to ensuring the safety and well-being of all clients by implementing a stringent Employee Criminal Background Check and Exclusion Screening Policy. This policy is designed to uphold the highest standards of security within our organization by conducting comprehensive criminal background checks and exclusion screenings for all employees and contractors prior to their hiring. In adherence to OAC 340:100-3-34 and OAC Chapter 680:3-10, this policy ensures that individuals with disqualifying criminal backgrounds are not placed in positions where they may have access to vulnerable populations in residential habilitation centers. By establishing this policy, [AGENCY NAME] aims to promote a secure, trustworthy environment for clients, staff, and the organization as a whole, while fulfilling state and federal regulatory requirements. The successful implementation of this policy is vital to our mission of providing quality care in a safe environment.

Procedures

- **Obtain Written Consent:**
  - The HR Manager shall obtain written consent from the applicant for background checks before conducting any checks.
  - Maintain the signed consent form as part of the employee's records.
- **Conduct Criminal Background Check:**
  - Within 48 hours of receiving consent, the HR Manager must conduct a criminal background check through state databases.
  - Document the results in the background check report.
- **Check Against Abuse Registries:**
  - Simultaneously with the criminal background check, the HR Manager shall verify the applicant against abuse registries and exclusion lists.
  - Document the results of the registry check as part of the hiring record.
- **Review and Evaluate Findings:**
  - Within 72 hours of receiving the background check results, the HR Manager and Administrator shall review and evaluate the findings.
  - Complete an evaluation report that summarizes the findings, which shall be retained in the employee's file for compliance.
- **Inform Applicant of Results:**
  - Within 5 business days of making a decision, the HR Manager must inform the applicant of the results and the decision made regarding their employment.
  - If applicable, provide an adverse action notice to the applicant, which must also be retained in their records.

- **Monitoring and Compliance:**
  - The Compliance Officer shall conduct regular audits of the background check process annually to ensure compliance with this policy.
  - Findings from these audits shall be reported to the Administrator, with immediate action taken to address any instances of non-compliance, including retraining staff as necessary.
- **Training Requirements:**
  - All new employees will receive initial training on the background check policy during orientation.
  - Annual refresher training on compliance and procedures will be provided to all staff as ongoing education.
  - Competency verification will occur through quizzes and practical evaluations to ensure understanding and compliance with the policy.
  - Maintain documentation of training attendance records and evaluation results for audit purposes.

References

- **OAC 340:100-3-34**: Oklahoma Administrative Code on Employee Criminal Background Checks (link)
- **OAC Chapter 680:3-10**: Oklahoma Administrative Code on Exclusion Screening (link)

Staff Credentialing and Competency Verification Policy

State: `OAC 340:100-3-39, OAC 340:100-5-35` **Federal:** `42 CFR 483.430(b)-(e)`

Policy Statement

The Staff Credentialing and Competency Verification Policy is designed to ensure that all personnel within [AGENCY NAME] operating in Oklahoma Residential Habilitation Centers meet the requisite educational, professional, and training standards as mandated by state and federal regulations. The primary objective of this policy is to enhance the safety, welfare, and quality of care provided to clients within these facilities. By establishing a structured framework for staff qualifications and ongoing professional competency, this policy serves to protect clients, improve care quality, and guarantee compliance with relevant regulations. [AGENCY NAME] is committed to implementing a comprehensive Staff Credentialing and Competency Verification Policy that will apply to all individuals providing services at Residential Habilitation Centers. This policy encompasses the full spectrum of staff management, including hiring practices, training protocols, performance evaluations, and continuous competency assessments. [AGENCY NAME] shall ensure that all staff possess the necessary qualifications, complete mandated training programs such as the Oklahoma DDS Foundations, and undergo regular competency evaluations to maintain their capability to provide effective care.

Procedures

- **Background Checks:**
  - Human Resources Manager shall conduct background checks for all potential staff members before hiring.
  - Documentation: Maintain background check records for review and compliance.
- **Qualifications Verification:**
  - Human Resources Manager shall verify the qualifications, educational background, and certifications of candidates prior to their hiring.
  - Documentation: Retain verification records to ensure compliance with credentialing standards.
- **Training Completion:**
  - Training Coordinator shall ensure that all new hires complete the Oklahoma DDS core training curriculum within 30 days of employment.
  - Documentation: Issue training completion certificates to track employee training status.
- **Annual Performance Evaluations:**
  - Department Supervisor shall conduct annual performance evaluations of staff to assess competency and adherence to protocols.
  - Documentation: Maintain performance evaluation reports for reference and accountability.
- **Ongoing Competency Assessments:**
  - Quality Assurance Manager shall document and review staff competency through quarterly training assessments.
  - Documentation: Keep competency assessment records to monitor ongoing staff development.

- Monitoring: Quality Assurance Manager will perform regular audits and compliance checks on a quarterly basis, reporting findings to the agency director and state regulatory bodies.
- Corrective Action: A plan to address any identified deficiencies will be developed within 30 days of a compliance review.

- **Training Requirements:**
  - All new hires must complete the Oklahoma DDS Foundations training within 30 days of commencing employment to ensure foundational knowledge.
  - Annual refresher training is required to maintain competencies, reinforcing essential skills and knowledge among staff.
  - Competency Verification: Staff capabilities will be assessed through hands-on evaluations and regular performance reviews.
  - Documentation: Training records must be rigorously maintained for a minimum of 5 years, ensuring accessibility for audits and compliance checks.

References

- **OAC 340:100-3-39**: Staff Credentialing Requirements
- **OAC 340:100-5-35**: Staff Training and Competency Verification
- **42 CFR 483.430**: Federal Regulations on Staff Qualifications

Quality Policies

Section Overview

The "Quality Policies" section of the Oklahoma Residential Habilitation Centers Policy & Procedure Manual encompasses essential guidelines designed to ensure regulatory compliance and uphold the highest standards of care. The Performance and Program Evaluation Policy focuses on continuous quality improvement through regular assessments of program effectiveness, while the Resident and Family Grievance and Ombudsman Access Policy safeguards residents' rights by providing transparent mechanisms for voicing concerns. These policies are critical for agencies operating residential habilitation centers, as they not only facilitate adherence to state and federal regulations but also enhance the overall quality of services provided. Moreover, they play a vital role in fostering trust and accountability, ultimately supporting the center's efforts for successful licensing and accreditation outcomes.

Performance and Program Evaluation Policy

State: `OAC 340:100-5-50` **Federal:** `42 CFR 483.440(c)`

Policy Statement

The purpose of the Performance and Program Evaluation Policy is to establish comprehensive procedures for evaluating the performance and efficacy of programs within Oklahoma Residential Habilitation Centers. This policy ensures compliance with both state and federal regulations while enhancing the quality of care provided to residents. It is vital for safeguarding resident rights and fostering a culture of accountability and continuous improvement, ultimately benefiting the residents, staff, and the broader organizational structure. [AGENCY NAME] is committed to maintaining an organized governance structure to promote the safety and welfare of clients. In fulfilling this commitment, [AGENCY NAME] shall implement a robust performance evaluation framework that covers all operational activities relating to program assessment. This framework will involve regular assessments of service outcomes, adherence to applicable regulations, and meticulous documentation to facilitate ongoing program development and enhancement.

Procedures

- **Performance Evaluation:**
  - The Quality Assurance Manager shall conduct a performance evaluation of all programs every six months. A comprehensive evaluation report documenting findings and recommendations will be prepared and maintained.
- **Client Progress Documentation:**
  - Direct Care Staff shall document individual client progress based on their Individualized Program Plan (IPP) on a monthly basis. Monthly progress notes for each client will be compiled and retained for review.
- **Resident Feedback Review:**
  - The Clinical Manager is responsible for reviewing resident feedback and complaints as part of the evaluation process on a monthly basis. A summary report of resident feedback and complaints will be generated and used to inform practice improvements.
- **Administrator Responsibilities:**
  - The Administrator shall approve this policy and any amendments, ensure compliance with state and federal regulations, and review annual evaluation reports for continuous oversight.
- **Clinical Management:**
  - The Clinical Manager shall oversee the implementation of performance evaluation activities and supervise direct care staff compliance, ensuring all evaluations reflect the true condition and needs of residents.
  - The Clinical Manager will also analyze resident feedback and utilize findings to enhance service delivery and care practices.
- **Staff Training:**
  - All staff must complete initial training on performance evaluations during their orientation to ensure understanding of policies and protocols.

- Annual ongoing training will be required to cover updated evaluation practices and compliance requirements, and staff must demonstrate their understanding through assessments and practical evaluations.
- Documentation of training records must be maintained in the individual staff files for future reference and compliance verification.

- **Compliance Monitoring:**
  - The Quality Assurance Department shall conduct regular audits and internal reviews of performance evaluations and documentation on a quarterly basis to ensure adherence to policy directives.
  - All findings from the audits must be reported to the Administrator and Clinical Manager, and any identified non-compliance will prompt the implementation of a corrective action plan to address deficiencies.

References

- **OAC 340:100-5-50**: Oklahoma Administrative Code 340:100 (link)
- **42 CFR 483.440(c)**: Federal Regulation 42 CFR 483.440 (link)

Resident and Family Grievance and Ombudsman Access Policy

State: `OAC 340:100-3-34, OAC 340:100-5-58.1-12` **Federal:** `42 CFR 483.420`

Policy Statement

The purpose of the Resident and Family Grievance and Ombudsman Access Policy is to establish a robust and transparent procedure that empowers residents and their families to submit complaints and grievances regarding the services provided at Oklahoma's residential habilitation centers. This policy is crucial for safeguarding client rights and creating a safe space where concerns can be expressed without fear of retaliation. By implementing this policy, [AGENCY NAME] is committed to enhancing the quality of care and services offered, ensuring compliance with state and federal regulations, and promoting open communication among residents, families, and staff. [AGENCY NAME] shall provide clear and accessible information regarding the grievance process, including details on how to contact the Ombudsman. This policy applies to all clients, their families, and staff engaged in the grievance process. The agency is committed to complying with all relevant regulations, including OAC 340:100-3-34, OAC 340:100-5-58.1-12, and 42 CFR 483.420. Key obligations under this policy include responding to grievances in a timely manner, ensuring residents have access to Ombudsman services, preventing retaliation against complainants, and meticulously documenting all grievances and the actions taken in response.

Procedures

- **Initial Information Provision:** Direct Care Staff shall provide residents and their families with information about the grievance process during the admission process. This must occur within 24 hours of admission and be documented using the Grievance Process Acknowledgment Form.
- **Grievance Acceptance:** Direct Care Staff shall accept grievances either verbally or in writing from residents and families as they are received. This should be recorded in the Grievance Log.
- **Grievance Acknowledgment:** Upon receipt of a grievance, the Clinical Manager shall acknowledge its receipt to the complainant within 48 hours. This acknowledgment must be documented using the Acknowledgment of Grievance Form.
- **Investigation:** The Administrator shall conduct a thorough investigation into the grievance within five business days of acknowledgment. The findings should be documented in an Investigation Report.
- **Outcome Communication:** Following the investigation, the Administrator shall provide a written response to the complainant regarding the outcome of the grievance within ten business days of receiving the grievance. This response must be documented using the Response to Grievance Form.
- **Ombudsman Notification:** At the time of providing the grievance outcome, Direct Care Staff shall inform the complainant of their right to contact the Ombudsman. This information should be provided using the Ombudsman Contact Information Sheet.
- **Documentation Maintenance:** All records related to grievances, including the Grievance Log, Investigation Reports, and Responses to Grievances, must be securely stored for a retention period of five years in a confidential location accessible only to authorized personnel. Documentation must be made visible for residents within the agency.
- **Compliance Monitoring:** The Quality Assurance Department shall conduct regular audits of grievance documentation and compliance with this policy on a quarterly basis. Findings from these

audits will be reported to the Administrator and reviewed in subsequent team meetings, with an established corrective action plan for any identified non-compliance.

- **Staff Training:** All staff must undergo initial training on grievance procedures during the onboarding process. Annual refresher training will be mandatory to ensure ongoing compliance and understanding. Employee competency will be assessed through regular evaluations and feedback, and training records will be maintained for all staff members to demonstrate compliance with these requirements.

References

- **OAC 340:100-3-34**: Oklahoma Administrative Code: Developmental Disabilities Services (link)
- **OAC 340:100-5-58.1-12**: Oklahoma Administrative Code: Grievance and Ombudsman Access Policy (link)
- **42 CFR 483.420**: Code of Federal Regulations: Grievance Process (link)

Safety & Emergency Policies

Section Overview

The "Safety & Emergency Policies" section outlines crucial guidelines designed to ensure the safety and well-being of residents in Oklahoma Residential Habilitation Centers. These policies serve the dual purpose of regulatory compliance and fostering a secure environment by addressing key focus areas such as fire safety, environmental safety, incident reporting, resident security, and emergency preparedness. Each policy is crafted to not only safeguard residents but also to facilitate meaningful responses to emergencies and unforeseen situations. Adherence to these policies is essential for maintaining licensing and accreditation, demonstrating our commitment to high standards of care, and promoting a culture of safety that prioritizes the needs and rights of residents.

Fire Safety and Life Safety Code Compliance Policy

State: `OAC 340:100-3-18, OAC Chapter 680` **Federal:** `42 CFR 483.470(i)`

Policy Statement

[AGENCY NAME] is committed to ensuring compliance with all applicable state and federal fire safety regulations to foster a secure environment for residents and staff alike. This policy is established not only to meet regulatory requirements but also to enhance the overall safety and well-being of individuals with developmental disabilities. Our goal is to protect residents, staff, and visitors by implementing proactive fire safety measures and procedures. To uphold a high standard of care and safety, [AGENCY NAME] shall prioritize regular training, thorough risk assessments, and effective emergency response protocols. This Fire Safety and Life Safety Code Compliance Policy applies to all facilities and personnel associated with the residential habilitation centers.

Procedures

- Conduct a comprehensive fire risk assessment annually.
  - **Responsibility:** Safety Officer
  - **Timing:** Annually, on or before [EFFECTIVE DATE]
  - **Documentation:** Record of risk assessment findings must be maintained for review.
- The Administrator shall approve the fire safety and life safety compliance policy and ensure adherence to all relevant regulatory standards.
- The Clinical Manager is to supervise safety training for staff and monitor compliance during inspections to ensure our practices align with safety expectations.
- Supervisors will implement daily safety checks and conduct fire drills to ensure staff compliance with safety protocols.
  - **Documentation:** Fire drill reports must be maintained.
- All Direct Care Staff must participate in fire safety training upon hire. To maintain competency, refresher training shall be provided every six months.
  - **Competency Verification:** Staff competency will be assessed through drills and evaluations.
  - **Documentation:** Training attendance records are required for all staff and must be securely stored.
- Conduct regular audits and safety inspections to monitor compliance, as follows:
  - **Frequency:** Quarterly
  - **Responsibility:** Safety Officer
  - **Reporting:** Audit results shall be reported to the Administrator on a quarterly basis.
  - **Corrective Action:** Immediate action will be taken to address any non-compliance identified during these reviews.
- All records related to fire safety, including fire drill reports, training attendance records, and inspection reports, must be retained for a period of five years.
  - **Storage:** All records shall be kept in a secure, confidential location to protect sensitive information.

Physical Plant and Environmental Safety Policy

State: `OAC 340:100-5-40, OAC Chapter 680` **Federal:** `ADA Title III`

Policy Statement

[AGENCY NAME] shall create and maintain a comprehensive Physical Plant and Environmental Safety Policy to ensure a safe and sanitary environment for all residents. This policy is established in compliance with state regulations (OAC 340:100-5-40, OAC Chapter 680) and federal standards under the Americans with Disabilities Act Title III. The policy aims to protect residents, staff, and visitors through adherence to safety regulations, fostering a culture of safety, and promoting overall quality and accessibility in facilities serving individuals with developmental disabilities. Compliance with this policy is mandatory for all residential habilitation centers operated in Oklahoma. This policy applies to all staff and operations within [AGENCY NAME]. All employees are required to familiarize themselves with these protocols to ensure a safe living and working environment that meets both state and federal guidelines. [AGENCY NAME] shall ensure that staff responsibilities include adherence to these standards, regular staff training, timely reporting of safety concerns, and participation in compliance monitoring activities.

Procedures

- Conduct monthly safety inspections of the physical plant, including fire alarms and emergency exits. The Facility Manager shall oversee these inspections, ensuring the completion of a safety inspection report each month.
- Provide ongoing training on emergency preparedness and physical plant safety protocols to all staff. The Training Coordinator is responsible for conducting this training annually and upon hiring, maintaining a training attendance record.
- Implement emergency drills, including fire, evacuation, and lockdown protocols on a quarterly basis. The Safety Officer shall oversee these drills and keep drill participation records for accountability.
- Ensure that all incidents and corrective actions taken are documented immediately following any occurrence. All staff are responsible for completing an incident report that captures the necessary details of the event.
- Conduct annual compliance assessments to ensure adherence to state and federal safety regulations. The Quality Assurance Manager shall carry out these assessments and maintain a compliance assessment report.
- Perform internal audits and inspections quarterly to verify compliance with safety protocols and state regulations. The Quality Assurance Manager will lead these audits, reporting results to the administration and staff, and developing action plans to address any identified non-compliance issues.
- All staff must complete safety and emergency preparedness training prior to direct assignment. Continuing education shall include annual refresher training on safety procedures and updates to regulations. Documentation of all training sessions and participant attendance shall be securely maintained.
- Verify staff competency through assessments and drills, ensuring all personnel are equipped to handle safety protocols effectively.
- Maintain records for a retention period of five years, following Oklahoma standards. All documentation, including safety inspection reports, training attendance records, incident reports,

and compliance assessment reports, must be stored securely with limited access to authorized personnel only.

References

- **OAC 340:100-5-40**: Oklahoma Administrative Code for Developmental Disabilities Services (link)
- **ADA Title III**: Americans with Disabilities Act Title III Regulations (link)

Incident Reporting and Critical Incident Review Policy

State: `OAC 340:100-3-27, OAC Chapter 680` **Federal:** `42 CFR 483.420(d)`

Policy Statement

The purpose of this Incident Reporting and Critical Incident Review Policy is to establish a systematic approach for the reporting and review of incidents that occur within Oklahoma's Residential Habilitation Centers. This policy exists to ensure compliance with state and federal regulations while promoting safety, quality care, and accountability in the services provided to residents. It aims to protect residents, staff, and the organization by facilitating timely reporting, thorough reviews, and continuous improvement of care practices. [AGENCY NAME] is committed to instituting a standardized protocol for reporting incidents, including accidents, injuries, abuse, and other critical events. In alignment with OAC 340:100-3-27 and 42 CFR 483.420(d), [AGENCY NAME] will ensure that effective incident reporting systems are in place. All staff members involved in the direct care and oversight of residents are required to adhere to this policy. Furthermore, [AGENCY NAME] shall provide comprehensive training to employees regarding their responsibilities under this policy, ensuring consistent documentation of incidents and implementing corrective actions based on the outcomes of incident reviews.

Procedures

- **Incident Reporting:**
  - Direct care staff must immediately report all critical incidents to the supervising manager within one hour of the incident. Documentation must include the completion of an Incident Report Form (IRF).
- **Supervisor Review:**
  - The supervising manager will review the IRF for completeness within 24 hours of submission. A Supervisor's Review Checklist (SRC) must also be completed during this review.
- **Submission for Further Review:**
  - Supervisors will submit the completed IRF and SRC to the Clinical Manager within 48 hours of the incident for further evaluation. Documentation of this submission must be maintained.
- **Critical Incident Review:**
  - The Incident Review Committee will conduct a Critical Incident Review within seven days of the incident. A Critical Incident Review Report (CIRR) must be generated documenting the findings and recommendations.
- **Implementation of Corrective Actions:**
  - The Clinical Manager is responsible for implementing corrective actions as recommended in the CIRR within 14 days of the review meeting. An Action Plan and Follow-up Report must be created to document these actions.
- **Ongoing Compliance Monitoring:**
  - The Quality Assurance Officer shall conduct regular audits of incident reports and review outcomes quarterly to monitor compliance with this policy. Findings will be reported to the Executive Leadership Team for further action.

- Any identified deficiencies will necessitate immediate corrective action to ensure adherence to incident management protocols.

- **Training Requirements:**
  - All direct care staff must receive initial training on incident reporting and critical incident review procedures during their orientation.
  - Annual refresher training on incident reporting and response protocols will be provided, with competency verified through simulated incident scenarios during these training sessions.
  - Documentation of training attendance records and competency evaluation results must be maintained for all staff members.

References

- **OAC 340:100-3-27**: Oklahoma Administrative Code for Incident Reporting (link)
- **42 CFR 483.420(d)**: Federal Regulation for Incident Reporting in RHCs (link)

Resident Security and Missing Person Policy

State: `OAC 340:100-3-27` **Federal:** `42 CFR 483.420(d)`

Policy Statement

This policy exists to ensure the safety and security of residents of Oklahoma Residential Habilitation Centers, as mandated by state and federal regulations. It establishes clear procedures for preventing elopement and responding effectively to incidents involving missing persons. The commitment of [AGENCY NAME] is to protect residents, empower staff with actionable steps, and comply with OAC 340:100-3-27 and 42 CFR 483.420(d). To fulfill this commitment, [AGENCY NAME] will implement stringent security measures, provide essential training to all staff members, maintain efficient communication with law enforcement, and keep accurate and timely records regarding resident safety and incidents. This policy applies to all staff members, volunteers, and contractors within [AGENCY NAME].

Procedures

- Conduct a daily security check to assess the safety of all residents.
  - **Responsible:** Direct Care Staff
  - **Timing:** Daily at the beginning of each shift
  - **Documentation:** Daily Security Check Log must be completed and stored securely.
- Immediately report any resident suspected to be missing to management and local law enforcement.
  - **Responsible:** Direct Care Staff
  - **Timing:** Immediately upon suspicion of a missing person
  - **Documentation:** Missing Person Incident Report must be filed.
- Initiate the emergency response plan and commence search efforts.
  - **Responsible:** Administrator/Director
  - **Timing:** Within minutes of a reported missing person
  - **Documentation:** Emergency Response Action Log shall be filled out alongside the search efforts.
- Notify the families of missing residents.
  - **Responsible:** Clinical Manager/Director of Nursing
  - **Timing:** Within one hour of confirming a missing person
  - **Documentation:** Family Notification Log must be maintained to track updates provided.
- Conduct internal audits and compliance checks to ensure adherence to the policy.
  - **Method:** Quality Assurance Team will perform audits
  - **Frequency:** Quarterly
  - **Reporting:** Results will be submitted to the Administrator for review, and corrective action plans will be implemented for identified deficiencies.
- Provide initial training on security procedures and response protocols to all staff upon hiring.
  - **Responsible:** Clinical Manager

    - **Documentation:** Training Attendance Records must be maintained for verification.

- Conduct annual refresher training to reinforce the policies and procedures established in the security protocol.
    - **Competency Verification:** Competency will be assessed through simulations and practical evaluations during training sessions.
    - **Documentation:** Training records shall be kept for all staff for a minimum of 5 years.

- Ensure secure storage of all documentation related to security measures and incidents.
    - **Storage:** All records, including Security Check Logs, Incident Reports, and Emergency Response Action Logs, must be securely stored in locked files with limited access.

References

- **OAC 340:100-3-27**: Oklahoma Administrative Code on Resident Security and Missing Person Policy (link)
- **42 CFR 483.420(d)**: Federal Regulation on Conditions of Participation for ICF/IIDs (link)

Emergency Preparedness and Disaster Planning Policy

State: `OAC 340:100-3-18` **Federal:** `42 CFR 483.470(i)`

Policy Statement

[AGENCY NAME] is committed to maintaining a safe environment for residents, staff, and visitors through the development and implementation of a comprehensive Emergency Preparedness and Disaster Planning Policy. This policy is designed to ensure compliance with all relevant state and federal regulations while effectively safeguarding vulnerable populations in residential habilitation centers. The goal is to facilitate quick responses during emergencies, maintain continuous operations amid disasters, and ultimately ensure the safety and security of all stakeholders. In accordance with this policy, [AGENCY NAME] shall take proactive measures to prepare for and respond to potential emergencies. This includes conducting regular assessments of risks and hazards, providing ongoing training for staff, and engaging residents in emergency preparedness drills. By establishing clear procedures and documentation requirements, [AGENCY NAME] will be well-equipped to manage emergencies effectively while fostering a culture of safety throughout the organization.

Procedures

- Develop a comprehensive emergency preparedness plan that includes evacuation routes, emergency contact numbers, and specific procedures for various types of emergencies. **Responsible:** Administrator/Director **Timing:** Within 30 days of policy effective date **Documentation:** Emergency Preparedness Plan document
- Conduct a risk assessment to identify potential hazards relevant to the facility and population served. **Responsible:** Clinical Manager/Director of Nursing **Timing:** Annually or as needed **Documentation:** Risk Assessment Report
- Schedule regular training sessions for staff on emergency procedures and specific roles during an emergency. **Responsible:** Supervisors **Timing:** Quarterly **Documentation:** Training Attendance Record
- Engage residents in emergency preparedness drills to familiarize them with the procedures and reduce anxiety. **Responsible:** Direct Care Staff **Timing:** Biannually **Documentation:** Drill Participation Log
- Establish communication protocols for notifying residents and contacting emergency services during an actual emergency. **Responsible:** Administrator/Director **Timing:** Within 14 days of plan development **Documentation:** Communication Protocol Document
- Implement a method for conducting audits and reviews of the emergency preparedness plan and drills. **Responsible:** Clinical Manager/Director of Nursing **Frequency:** Quarterly **Report:** Monitoring results reported to agency administration **Corrective Action:** Develop action plans to address any identified non-compliance
- Review and revise the emergency preparedness plan and training materials based on feedback from drills and actual emergencies. **Responsible:** Administrator/Director **Timing:** Annually or after actual event **Documentation:** Revision Log
- Ensure that all staff receive initial training on emergency protocols during orientation and participate in ongoing annual refresher courses. **Documentation:** Training records maintained for audit purposes

- Verify competency through participation in drills and testing to ensure all staff members are adequately prepared to respond to emergencies. **Documentation:** Training records maintained

Appendix A: Required Forms Checklist

This checklist consolidates all forms referenced across the policies in this manual. Agencies should ensure these forms are developed, maintained, and readily accessible to staff.

Form Name	Purpose	Frequency	Related Policy
Acknowledgment of Grievance Form	To confirm receipt of a grievance from the resident/ family.	Upon receiving a grievance	`Resident and Family Grievance and Ombudsman Access Policy`
Annual Audit Report	To document the findings of the annual audit of resident funds accounts.	Annually	`Resident Funds and Personal Property Management Policy`
Assessment Form	To assess the therapy needs of the resident upon admission.	Once per admission	`Therapy and Clinical Services Policy`
Background check consent form	To authorize background checks for potential employees.	At hiring	`Governing Body and Administrative Oversight Policy`
Consent Form	To obtain resident's written acknowledgment of their rights.	During rights discussion	`Rights Notification and Documentation Policy`
Corrective Action Plan Template	For documenting responses to identified deficiencies	As needed	`Survey and Regulatory Oversight Compliance Policy`
			`Incident Reporting and`

Form Name	Purpose	Frequency	Related Policy
Critical Incident Review Report (CIRR)	To summarize findings from reviews of serious incidents.	As needed following a critical incident.	`Critical Incident Review Policy`
Data Retention Schedule Template	To document the retention periods for various types of records.	Reviewed annually	`Data Retention and Destruction Policy`
DDS Staff Training Record	To document staff training related to licensure and care requirements.	Upon completion of training programs.	`Licensure, Certification, and Contractual Compliance Policy`
Dietary Assessment Form	To assess the nutritional needs of residents.	Upon admission and annually thereafter.	`Nutrition, Dietary, and Food Service Policy`
Family Notification Log	To track communication with families regarding missing persons	Every time a notification is made	`Resident Security and Missing Person Policy`
Fire Safety Drill Log	To record fire drill dates, participants, and outcomes.	After each fire drill.	`Fire Safety and Life Safety Code Compliance Policy`
Food Safety Checklist	To ensure proper food handling and preparation procedures.	Daily.	`Nutrition, Dietary, and Food Service Policy`
Grievance Process Acknowledgment Form	To acknowledge that the resident/family has received information about the grievance process.	At admission	`Resident and Family Grievance and Ombudsman Access Policy`
HIPAA Privacy Breach Notification Form	Used to notify affected individuals of a breach of PHI.	As breaches occur.	`Health Information Privacy and`

Form Name	Purpose	Frequency	Related Policy
			`HIPAA Compliance Policy`
Incident Report Form	To record incidents involving clients or personnel.	As needed based on occurrences.	`Staff Training and Orientation Policy`
Incident report form	Document exposure incidents and responses.	Each time an incident occurs	`Exposure Control and Reporting Policy`
Incident Report Form (IRF)	To document details of incidents involving residents.	After each critical incident.	`Incident Reporting and Critical Incident Review Policy`
Incident Reporting Form	To document incidents and the subsequent actions taken.	As needed after incidents	`Physical Plant and Environmental Safety Policy`
Individual Program Plan (IPP) Template	To outline and document individual care plans.	At admission and updated at quarterly reviews	`Clinical Records and Documentation Policy`
Initial Financial Assessment Form	To document the initial financial assessment of the resident.	Once at admission	`Resident Funds and Personal Property Management Policy`
IPP Form	To document individualized therapy needs and plans for residents.	One-time at admission, updated as needed	`Therapy and Clinical Services Policy`
LTC-300	Application for services in Residential Habilitation Centers.	Once per applicant, at the	`Admission, Eligibility,`

Form Name	Purpose	Frequency	Related Policy
		time of admission.	`and Discharge Policy`
Medication Disposal Form	To ensure proper documentation and disposal of expired or unused medications.	Monthly or as needed.	`Medication Administration, Storage, and Management Policy`
Missing Person Incident Report	To document incidents of missing residents promptly and accurately	Every time a resident is reported missing	`Resident Security and Missing Person Policy`
Monthly Account Statement	To provide residents with an overview of their account status.	Monthly	`Resident Funds and Personal Property Management Policy`
OHCA Cost Report Form	To report costs associated with providing residential services.	Quarterly	`Billing, Claims, and Cost Reporting Policy`
Oklahoma Background Check Consent Form	To obtain written consent from applicants for the background check process.	Once per applicant.	`Employee Criminal Background Check and Exclusion Screening Policy`
Oklahoma Client Assessment Form	To document initial clinical assessments for residents.	At admission and on re-evaluation	`Clinical Records and Documentation Policy`
Oklahoma DDS Billing Claim Form	To submit claims for Medicaid reimbursement.	Per service delivered	`Billing, Claims, and Cost Reporting Policy`

Form Name	Purpose	Frequency	Related Policy
Oklahoma DDS core training form	Document completion of required training for staff.	Upon new hires and annual refreshers.	`Accreditation and Organizational Ethics Policy`
Oklahoma Employee Background Check Result Evaluation Form	To document the evaluation of results from background checks.	Once per applicant.	`Employee Criminal Background Check and Exclusion Screening Policy`
Oklahoma Form 340:100-001	Template for documenting an Individualized Program Plan.	Used at the development of the IPP and upon revision.	`Active Treatment and Habilitation Policy`
Oklahoma Form 340:100-002	Log for documenting daily activities and progress.	Used daily as part of active treatment documentation.	`Active Treatment and Habilitation Policy`
Oklahoma Form Emergency Preparedness Checklist	To ensure all required components of the emergency preparedness plan are addressed	Annually or when updates occur	`Emergency Preparedness and Disaster Planning Policy`
Oklahoma Form for Incident Reporting	Used to report any breaches of PHI.	As incidents occur.	`Health Information Privacy and HIPAA Compliance Policy`
Oklahoma Form Infection Control Log	To document infection control practices and compliance.	Used continuously during audits and training	`Standard and Transmission-Based Precautions Policy`

Form Name	Purpose	Frequency	Related Policy
Oklahoma Form IPP-1	To document individual program plans for clients.	At the beginning of each client's admission and reviewed annually.	`Performance and Program Evaluation Policy`
Oklahoma Form LIC-002	Used to acknowledge receipt of Residents' Rights.	At time of admission.	`Resident Rights and Protections Policy`
Oklahoma Form LIC-003	Used to report any incidents related to resident rights violations.	As incidents occur.	`Resident Rights and Protections Policy`
Oklahoma Form LIC-XXXX	To provide documentation of compliance with state regulations for staff qualifications.	Annually or as required	`Governing Body and Administrative Oversight Policy`
Oklahoma Form PR-2	To record progress for each client based on their IPP.	Monthly	`Performance and Program Evaluation Policy`
Oklahoma Form_MA_002 - Maintenance Checklist	To ensure all vehicles are maintained according to state regulations.	Monthly	`Transportation and Community Access Policy`
Oklahoma Form_TC_001 - Transportation Log	To document all transportation services provided to clients.	For each transportation event	`Transportation and Community Access Policy`
Oklahoma Immunization Consent Form	Document consent for immunizations administered to residents.	At admission and prior to any immunization administration.	`Immunization and Infectious Disease Policy`
Oklahoma Immunization Tracking Form	Track immunizations received by residents and staff.	Annually and upon receipt of	`Immunization and Infectious Disease Policy`

Form Name	Purpose	Frequency	Related Policy
		any immunization.	
Oklahoma Incident Report Form	To document incidents requiring restraint or seclusion.	As needed following incidents.	`Restraint, Seclusion, and Behavior Intervention Policy`
Oklahoma Licensure Application Form	To apply for the necessary licenses to operate a Residential Habilitation Center in Oklahoma.	Annually, or as required for renewals.	`Licensure, Certification, and Contractual Compliance Policy`
Oklahoma Medication Error Report Form	To document any medication errors that occur during administration.	As needed.	`Medication Administration, Storage, and Management Policy`
Oklahoma Notification of Restraint or Seclusion Form	To notify guardians of the use of restraint or seclusion.	Whenever restraint or seclusion is used.	`Restraint, Seclusion, and Behavior Intervention Policy`
Oklahoma Performance Evaluation Form	To document staff performance evaluations in compliance with state regulations.	Annually for each staff member	`Staff Supervision and Performance Evaluation Policy`
Oklahoma Reasonable Accommodation Request Form	To document requests for accommodations by residents.	As needed	`Americans with Disabilities Act (ADA) Accessibility Policy`
Oklahoma Staff Training Documentation	To record any training sessions attended by staff related to evaluation and performance.	As training occurs	`Staff Supervision and Performance`

Form Name	Purpose	Frequency	Related Policy
			`Evaluation Policy`
Oklahoma Staffing Ratio Reporting Form	To report staffing levels to state authorities.	Quarterly	`Staffing Plan and Sufficient Coverage Policy`
Ombudsman Contact Information Sheet	To provide residents with contact information for the Ombudsman.	At the time of response	`Resident and Family Grievance and Ombudsman Access Policy`
Personal Funds Account Setup Form	To establish a personal funds account for the resident.	Once at admission	`Resident Funds and Personal Property Management Policy`
Record Destruction Log	To record methods and dates of record destruction.	As records are destroyed	`Data Retention and Destruction Policy`
Resident Rights Acknowledgment Form	To inform residents of their rights upon admission.	Upon admission	`Rights Notification and Documentation Policy`
Response to Grievance Form	To communicate the outcome of the grievance investigation.	Upon completion of grievance investigation	`Resident and Family Grievance and Ombudsman Access Policy`
Safety Training Attendance Form	To record attendance at safety training sessions.	During each training session	`Physical Plant and Environmental Safety Policy`
Sanitation Inspection Report		Weekly.	`Nutrition, Dietary, and`

Form Name	Purpose	Frequency	Related Policy
	To document cleanliness and compliance of kitchen and dining areas.		`Food Service Policy`
Service Delivery Documentation Form	To record the nature and details of services provided.	After each service delivery	`Clinical Records and Documentation Policy`
Staff Training Record	To track training sessions and staff attendance.	Used annually or during new staff onboarding	`Standard and Transmission-Based Precautions Policy`
Staff Training Record Form	To track completed training and ongoing education.	Whenever training is completed.	`Staff Credentialing and Competency Verification Policy`
Survey Preparedness Checklist	To document readiness for compliance surveys	Each survey cycle	`Survey and Regulatory Oversight Compliance Policy`
Training Attendance Record	Track attendance for training sessions.	After each training session.	`Infection Prevention and Control Policy`
Training Completion Form	To document completion of training requirements by staff.	Upon completion of each training session.	`Staff Training and Orientation Policy`

Forms Development Notes

- All forms should include agency name, logo, and version/revision date
- Forms should be reviewed annually for regulatory compliance
- Maintain both electronic and paper versions as appropriate
- Train staff on proper completion and filing of all forms

Appendix B: Policy-to-Regulation Crosswalk

This crosswalk maps each policy to its applicable state regulations, federal requirements, and accreditation standards. Use this reference to ensure comprehensive regulatory compliance.

Policy	State Citations	Federal Citations	Accreditation Standards
Staff Training and Orientation Policy `OK-RHC-PER-003`	`OAC 340:100-3-34`, `OAC 340:100-5-35`	N/A	N/A
Governing Body and Administrative Oversight Policy `OK-RHC-ADM-001`	`OAC 340:100-3-2`	`42 CFR 483.410`	N/A
Health Information Privacy and HIPAA Compliance Policy `OK-RHC-ADM-005`	`OAC 340:100-3-39`	`45 CFR 160`, `45 CFR 164`	N/A
Fire Safety and Life Safety Code Compliance Policy `OK-RHC-SAF-004`	`OAC 340:100-3-18`, `OAC Chapter 680`	`42 CFR 483.470(i)`	N/A
Licensure, Certification, and Contractual Compliance Policy `OK-RHC-ADM-002`	`OAC 340:100-3-2`, `OAC 340:100-3-39`, `OAC Chapter 680`	`42 CFR 483.410`	N/A
Physical Plant and Environmental Safety Policy `OK-RHC-SAF-003`	`OAC 340:100-5-40`, `OAC Chapter 680`	`ADA Title III`	N/A
Quality Assurance and Performance Improvement (QAPI) Policy `OK-RHC-ADM-006`	`OAC 340:100-3-27`	`42 CFR 483.475`	N/A

Policy	State Citations	Federal Citations	Accreditation Standards
HCBS Settings Compliance Policy `OK-RHC-ADM-011`	`OAC 317:40`	`42 CFR 441.301`	N/A
Nutrition, Dietary, and Food Service Policy `OK-RHC-CLI-008`	`OAC 340:100-5-30`, `OAC 340:100-5-31`	`42 CFR 483.480`	N/A
Data Retention and Destruction Policy `OK-RHC-DOC-003`	`OAC 340:100-3-39`	`45 CFR 164.530`	N/A
Staff Supervision and Performance Evaluation Policy `OK-RHC-PER-004`	`OAC 340:100-3-34`	`42 CFR 483.430(e)(1)-(2)`	N/A
Medication Administration, Storage, and Management Policy `OK-RHC-CLI-006`	`OAC 340:100-5-52`, `OAC 340:100-5-32`	`42 CFR 483.460`	N/A
Billing, Claims, and Cost Reporting Policy `OK-RHC-FIN-001`	`OAC 340:100-3-60`, `OHCA Cost Report Guidelines`	N/A	N/A
Active Treatment and Habilitation Policy `OK-RHC-CLI-002`	`OAC 340:100`, `OAC 317:40`	`42 CFR 483.440`	N/A
Immunization and Infectious Disease Policy `OK-RHC-INF-002`	`OAC 340:100-3-22.1`	`CMS Infection Control`	N/A
Resident Funds and Personal Property	`OAC 340:100-3-8.1`	`42 CFR 483.420(a)(5)`	N/A

Policy	State Citations	Federal Citations	Accreditation Standards
Management Policy `OK-RHC-ADM-008`			
Infection Prevention and Control Policy `OK-RHC-INF-001`	`OAC 340:100-3-22.1`	`42 CFR 483.470(i)`	N/A
Resident Funds Protection and Management Policy `OK-RHC-FIN-002`	`OAC 340:100-3-8.1`	`42 CFR 483.420(a)(5)`	N/A
Restraint, Seclusion, and Behavior Intervention Policy `OK-RHC-CLI-005`	`OAC 340:100-5-57`	`42 CFR 483.430(e)`	N/A
Individual Program Plan (IPP) and Person-Centered Planning Policy `OK-RHC-CLI-001`	`OAC 340:100-5, OAC 317:40`	`42 CFR 483.440`	N/A
Performance and Program Evaluation Policy `OK-RHC-QUA-001`	`OAC 340:100-5-50`	`42 CFR 483.440(c)`	N/A
Exposure Control and Reporting Policy `OK-RHC-INF-004`	`OAC 340:100-3-22.1`	N/A	`OSHA Bloodborne Pathogens Standard` `ACHC/CHAP`
Consent for Care and Treatment Policy `OK-RHC-CLI-004`	`OAC 340:100-5-22.1`	`42 CFR 483.420(a)(2)-(3)`	N/A

Policy	State Citations	Federal Citations	Accreditation Standards
Resident Rights and Protections Policy `OK-RHC-CLI-003`	`OAC 340:100-5-22.1, OAC Chapter 680`	`42 CFR 483.420`	N/A
Therapy and Clinical Services Policy `OK-RHC-CLI-007`	`OAC 340:100-5-22, OAC 317:30-5-95`	`42 CFR 483.440(b)-(d)`	N/A
Resident and Family Grievance and Ombudsman Access Policy `OK-RHC-QUA-002`	`OAC 340:100-3-34, OAC 340:100-5-58.1-12`	`42 CFR 483.420`	N/A
Americans with Disabilities Act (ADA) Accessibility Policy `OK-RHC-ADM-009`	`Oklahoma Technical Assistance Document Section 508`	`ADA Title III`	N/A
Standard and Transmission-Based Precautions Policy `OK-RHC-INF-003`	`CMS Infection Control`	N/A	`CMS Infection Control` `ACHC/CHAP`
Accreditation and Organizational Ethics Policy `OK-RHC-ADM-010`	`ACHC/CHAP Standards`	N/A	`ACHC/CHAP Standards`
Survey and Regulatory Oversight Compliance Policy `OK-RHC-ADM-007`	`OAC Chapter 680:3-9`	N/A	`CMS State Operations Manual Appendix J`
Incident Reporting and Critical Incident Review	`OAC 340:100-3-27, OAC Chapter 680`	`42 CFR 483.420(d)`	N/A

Policy	State Citations	Federal Citations	Accreditation Standards
Policy OK-RHC-SAF-002			
Admission, Eligibility, and Discharge Policy OK-RHC-ADM-003	OAC 340:100-5-3, OAC 340:100-3-2, OAC 317:30-5-122, OAC 317:35-9-45	42 CFR 483.440	N/A
Staffing Plan and Sufficient Coverage Policy OK-RHC-PER-005	OAC 340:100-3-34, OAC 340:100-5-35	42 CFR 483.430(d)	N/A
Rights Notification and Documentation Policy OK-RHC-DOC-002	OAC 340:100-5-22.1, OAC Chapter 680	N/A	N/A
Employee Criminal Background Check and Exclusion Screening Policy OK-RHC-PER-001	OAC 340:100-3-34, OAC Chapter 680:3-10	N/A	N/A
Transportation and Community Access Policy OK-RHC-CLI-009	OAC 340:100-5-33.1, OAC 317:30-5-95.59	N/A	N/A
Clinical Records and Documentation Policy OK-RHC-DOC-001	OAC 340:100-3-39	42 CFR 483.420(c)	N/A
Staff Credentialing and Competency Verification Policy OK-RHC-PER-002	OAC 340:100-3-39, OAC 340:100-5-35	42 CFR 483.430(b)-(e)	N/A
Resident Security and Missing	OAC 340:100-3-27	42 CFR 483.420(d)	N/A

Policy	State Citations	Federal Citations	Accreditation Standards
Person Policy `OK-RHC-SAF-005`			
Posting of Compliance and License Documentation Policy `OK-RHC-ADM-004`	`OAC 340:100-3-34, OAC Chapter 680:3-10, OAC Chapter 680:3-11`	N/A	N/A
Emergency Preparedness and Disaster Planning Policy `OK-RHC-SAF-001`	`OAC 340:100-3-18`	`42 CFR 483.470(i)`	N/A

Regulatory Notes

- **State Regulations:** Oklahoma state-specific requirements for Residential habilitation centers
- **Federal Regulations:** Medicare Conditions of Participation (42 CFR Part 484)
- **Accreditation:** Standards from ACHC, CHAP, Joint Commission, and other accrediting bodies

Note: Regulations are subject to change. Verify current requirements with regulatory agencies.